

UMOJA PEOPLE WITH DISABILITIES



**REPORT ON THE SITUATION ANALYSIS OF SEXUAL AND
REPRODUCTIVE HEALTH RIGHTS AND NEEDS OF WOMEN AND
GIRLS WITH DISABILITIES AT DZALEKA REFUGEE CAMP, MALAWI**

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Submitted to :

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Acronyms and abbreviations

ART	Anti-retroviral therapy
CBO	Community Based organisation
FGD	Focus Group Discussions
HIV	Human Immunodeficiency Virus
KII	Key Informant Interviews
NGO	Non - Governmental Organisation
PWD	Persons with disabilities
SBCC	Social Behavioural Change Communication
SDPs	Service Delivery Points
SRHR	Sexual and Reproductive Health and Rights
STI's	Sexually transmitted infections
UNCRPD	United Nations Convention on the Rights of Persons with Disabilities
UNHCR	United Nations Commissioner for Refugees
UPD	Umoja People with Disabilities
WHO	World Health Organisation
WGSS	Washington Group of Disability Statistics

Executive summary

Background information

Umoja People with Disabilities (UPD) commissioned a study to explore the specific sexual and reproductive health and rights (SRHR) needs and barriers faced in accessing SRHR services by women and girls with disabilities at Dzaleka Refugee Camp in Malawi. The study aimed at understanding the intersection of disability, gender, and refugee status in influencing access to sexual and reproductive health services (SRHR) and affecting overall health outcomes.

Study approach

The study employed a mixed method approach where both quantitative and qualitative methods were used. A survey and focus group discussions were further employed to get a deeper understanding of the issues on access, and barriers on SRHR of women and adolescent girls with disabilities at Dzaleka Refugee Camp. The study also engaged key informants from law enforcement agencies such as the Police and other stakeholders working on women's rights issues such as Plan Malawi.

Summary of Findings

The findings emerging from the study are summarised as follows;

- The survey found that there are significant gaps in knowledge of SRHR particularly among younger women and girls. However, there is high knowledge of SRHR among older women from the age of 36 and above.
- The survey found out that friends and peers are the most significant source of SRHR information in the camp. This highlights the significance of social networks in shaping health seeking behaviours such as of access and utilisation of health services among girls and women with disabilities.
- On the most accessible SRHR service, the analysis indicates that HIV testing and counselling stands out as the most commonly accessible service, with 87% of respondents reporting

availability. The second most utilised service was family planning, especially among older women.

- The survey found out that the government is the major provider of sexual and reproductive health services. This conclusion is deduced from the availability of a local health centre within the camp believed to be the only one providing free SRHR. Notwithstanding the above, there was a clear gap on service providers.
- The gap in service delivery has opened up space to traditional practitioners such as traditional birth attendants to become a reliable source of abortion services which are mostly unsafe as reported by girls and women with disabilities.
- While assessing the attitude and perception of the respondents regarding sexual reproductive health services, the study found that the community does not encourage and expect girls with disabilities to seek SRHR services. This has necessitated an increase in cases of STIs and unplanned pregnancies among girls and women with disabilities.
- On barriers to access of SRHR, the study concludes that long distance, unfriendly attitudes of health workers, and community stigma all interact to hinder girls and women with disabilities from accessing SRHR. For instance, health personnel tend to antagonize women with disabilities when accessing SRHR services particularly maternity services as they are deemed not having the capacity to bear children.
- There is high sexual activity among girls and women with disabilities despite there being an inconsistent condom use. 79% of individuals with disabilities reported to be sexually active while only 33% acknowledged to be using condoms during sex.
- The study found that there are alarming rates of abusive behavior in relationships, particularly highlighting the heightened vulnerability faced by individuals with disabilities. The cases of abuse included emotional, physical and sexual abuse committed against girls and women individuals with disabilities.
- Most women and girls with disabilities in the camp are structurally impeded from utilising the available justice system in the face of domestic and sexual violence due to impending resettlement programs. In the end, they are denied access to justice because they face multiple barriers when attempting to assert their rights in local justice systems

Recommendations

Knowledge and sources of information on Sexual and Reproductive Health i.e HIV & AIDS and Sexually Transmitted Infections (STI's)

- There is a need for enhancing outreach efforts that build on peer networks and community health initiatives to improve SRHR knowledge and resources within the camp. It is therefore recommended that Umoja Peoples with Disabilities (UPD) should develop a comprehensive and inclusive sexual reproductive health education guide and recruit community volunteers who can provide outreach education services, targeting girls and women with disabilities in the community.

Access to sexual and reproductive health services

- There is a need for targeted educational initiatives to enhance awareness of available SRHR services within the camp. Furthermore, it would be advisable for future SRHS in the camp to target the age bracket of girls and young women with disabilities.
- We therefore recommend that UPD should firstly, support the government health personnel at the hospital's youth friendly corner with training on inclusive sexual and reproductive health service delivery.
- Secondly, UPD should initiate community outreach clinics in collaboration with religious leaders, community elders, single mother's association and other structures in order to reach out to more girls and women with SRHS.
- UPD should also mobilise resources and open an affordable clinic within the community. The clinic should service all youth but with a strong focus on reaching out to girls and women with disabilities. Community volunteers could use the clinic as their reference station.

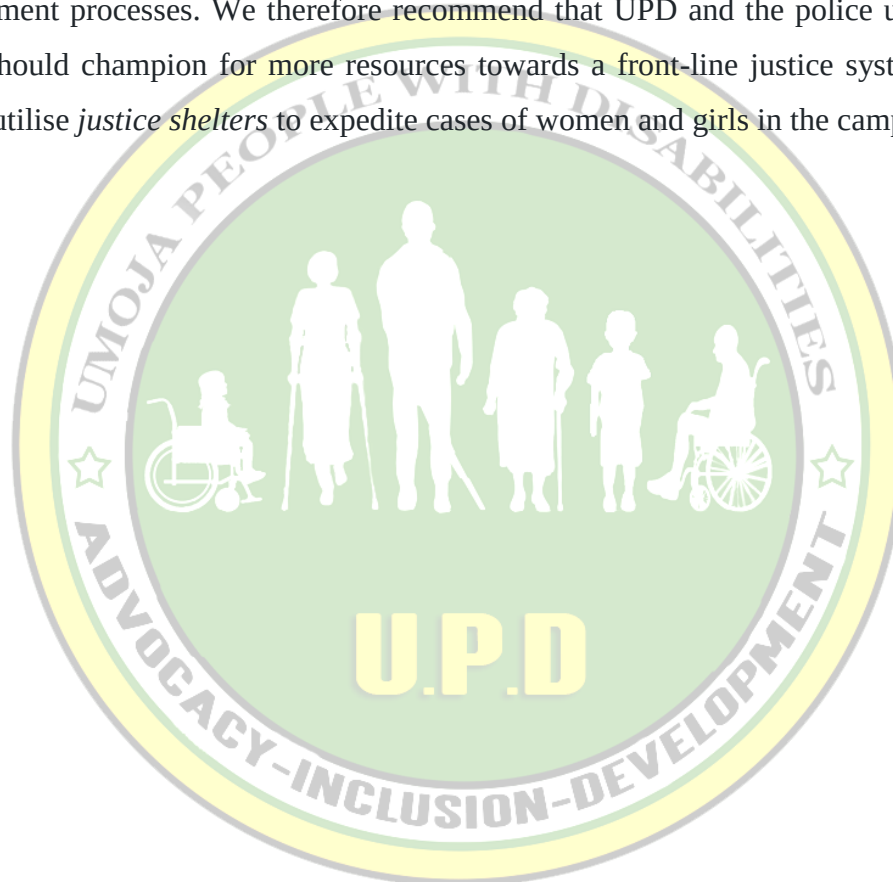
Attitudes, norms and practice on disability and SRHR

- Norms and cultural impressions have a continuous impact on issues of access to sexual and reproductive health services within the camp. We therefore recommend UPD with other stakeholders to develop a social behavioural change and communication strategy (SBCC) on SRHR issues with an intersectionality of gender and disability. This will help to influence social networks such as family and guardians and other informal structures that shape access to SRHR information and services.
- Increase the number of gatekeepers in the communities to assist in behaviour change in conservative camp communities. UPD should initiate the recruitment of traditional birth attendants, community leaders, and training them on SRHR.

- There is a need for the establishment of a camp health committee or technical working group involving all NGOs and CBOs operating in the camp. UPD could champion the cause for the protection of girls and women with disabilities at the technical group level to leverage on partnerships and resources

Violence and justice system

- Many girls and women with disabilities in the camp live with compounding “hidden” legal problems regarding sexual violence. As indicated in findings, women and girls are less likely to request or receive protection from law enforcement due to the fear of jeopardising their resettlement processes. We therefore recommend that UPD and the police unit located in the camp should champion for more resources towards a front-line justice system approach that would utilise *justice shelters* to expedite cases of women and girls in the camp.



1. Introduction and background

1.1. Background

A refugee is defined as a person who flees his or her country due to fear-based on factors such as persecution, gender, religion or race and who cannot return to his/her country based on those fears hence seeks protection in another country¹. By the end of 2024, there were about 32 million refugees under the United Nations High Commissioner for Refugees of which about half are women². Quality of life for refugees is generally low as most are hosted in refugee camps where there is usually a shortage of social services such as education, health and food as well as limitations in movement outside the camp and right to work³. As such, refugees have to rely on humanitarian agencies and institutions for the provision of basic needs.

Persons with disabilities in humanitarian settings are one of the most vulnerable groups as refugee status compounds their vulnerability⁴. However, they are often ignored in programming and accessing services. Within the broader context of refugees with disabilities, women and girls are often pushed to the margin because of the intersection of gender, disability and refugee status⁵.

Sexual and reproductive health rights are defined as a range of services such as maternal and neonatal health (including prevention and management of unsafe abortion), young people sexual and

¹ UN General Assembly, 1951 Convention Relating to the Status of Refugees, United Nations, Treaty Series, vol. 189, p. 137, 28 July 1951, <https://www.refworld.org/legal/agreements/unga/1951/en/39821> [accessed 04 April 2025]

² UNHCR, (2024). Figures at a glance available at <https://www.unhcr.org/about-unhcr/overview/figures-glance>

³ Papadopoulos, R. (2008). Systematic challenges in a refugee camp. *Context*, (99), 16-19. https://repository.essex.ac.uk/2040/1/RKP_Context_paper_08.pdf

⁴ Holden, J., Lee, H., Martineau-Searle, L., & Kett, M. (2019). Disability inclusive approaches to humanitarian programming: Summary of available evidence on barriers and what works (Disability Inclusion Helpdesk Research Report No. 9). Disability Inclusion Helpdesk at <https://assets.publishing.service.gov.uk/media/5d89f3e540f0b61c7ba3ba3b/query-9-evidence-humanitarian-response1.pdf>

⁵ Paik, K. 2014. *Strong Girls, Powerful Women. Program Planning and Design for Adolescent Girls in Humanitarian Settings*. New York: Women's Refugee Commission. <https://www.womensrefugeecommission.org/research-resources/strong-girls-powerful-women-report/>

reproductive health, family planning, prevention and management of Sexually Transmitted Infections (STI's), HIV and AIDS early detection and management of cervical, prostate and breast cancers, elimination of harmful maternal practices including domestic and sexual violence⁶. These services are aimed at ensuring that every person has a satisfying and safe sexual life. However, access to SRHR services is limited for persons with disabilities, particularly women due to prevailing cultural norms and other environmental barriers⁷.

The rights of persons with disabilities to health and participation in matters affecting their lives is anchored in the 2007 United Nations Convention on the Rights of Persons with Disabilities (UNCRPD). The UNCRPD recognises the multiple deprivations that for women with disabilities face hence makes provisions to ensure their full development, advancement and empowerment so that they enjoy their full rights and freedoms⁸.

1.2. Purpose of study

As it was aforementioned, the study aimed at determining how the intersection of gender and disability contributes to the challenge of accessing SRHR services. The study further aimed at illuminating how displacement and 'transitional' camp settlements shape issues of SRHR services for girls and women with disabilities

Specific Objectives of the study

The specific objectives of the study were to;

- i. Identify the specific SRHR needs of women and girls with disabilities, including but not limited to access to contraception, maternal health care, menstrual hygiene, prevention of sexual violence, and sexual health education.

⁶ Government of Malawi (2021). *National Sexual and Reproductive Health and Rights (SRHR) Policy*. Ministry of Health available at https://malawi.unfpa.org/sites/default/files/resource-pdf/Malawi_National_SRHR_Policy_2017-2022_16Nov17.pdf

⁷Ganle JK, Baatiema L, Quansah R, Danso-Appiah A. (2020) Barriers facing persons with disability in accessing sexual and reproductive health services in sub-Saharan Africa: A systematic review. PLoS One. 2020 Oct 12;15(10):e0238585. doi: 10.1371/journal.pone.0238585. PMID: 33044966; PMCID: PMC7549766.

⁸ UN General Assembly, Convention on the Rights of Persons with Disabilities : resolution / adopted by the General Assembly, A/RES/61/106, 24 January 2007, <https://www.refworld.org/legal/resolution/unga/2007/en/49751> [accessed 31 March 2025]

- ii. Assess the physical, social, and economic barriers these women and girls face in accessing SRHR services and information.
- iii. Evaluate the role of community attitudes, cultural norms, and institutional practices in shaping access to SRHR care for women and girls with disabilities.
- iv. Gather recommendations for improving SRHR service delivery and support for women and girls with disabilities at the camp

1.3. Methodology and ethical considerations

1.3.1. Study design

A convergent parallel mixed methods design was used for its time efficiency and ability to offer balanced insights. This involved collecting of quantitative and qualitative data concurrently and analysing it separately for a complementary picture of the SRHR needs.

1.3.2. Sampling procedures

The study combined both quantitative and qualitative sampling procedures. As such, purposive sampling was employed to recruit refugee women and adolescent girls with disabilities within the reproductive age of 15- 49. A random sampling procedure was adopted for quantitative data.

Table 1: Summary of study participants

The study engaged 63 participants of which 57 were women and girls with disabilities who were in a survey and focus group discussions. Six key informants also gave valuable information and these included Police, Plan Malawi, Women leaders from UPD, Health Personnel and a Single mother's organisation representative.

Figure 1: Summary of study participants

57 
Women and girls with disabilities

5 
Key Informants (Police - Female), UPD (Two Female) (Plan Malawi- Female); Single Mothers- Female)

1 
Health personnel (Male)- KIII

1.3.3. Data collection methods

Tool 1- Survey: A structured questionnaire was used to collect data on SRHR needs, access to services, barriers that are encountered, and satisfaction with existing services. The survey also collected data relating to variables like types of disabilities, knowledge and awareness of SRH services, SRH outcomes, nationalities, and discrimination experiences. 40 women and girls with disabilities were reached through quantitative surveys.

Tools 2- Focus Group Discussions: Data collection for the qualitative data involved multiple methods. Three focus group discussions were held covering 17 women and girls with disabilities. In setting the discussions, efforts were made to ensure that different disability types have been represented. During the discussions, lived experiences, perceptions, and suggestions for improving SRHR services were central topics.

A focus group discussion with women with disabilities



In-depth Interviews: Key Informant Interviews were conducted with healthcare providers around the camp, police UPD Women leaders, PLAN Malawi.

1.3.4. Data Management and Analysis

Quantitative Data: The survey questionnaire developed was deployed to Kobo Collect which expedited data collection and cleaning processes while ensuring that the assessment benefits from the advantages of computer-assisted data collection. The data was reviewed daily before being uploaded to the server. The data was extracted to Microsoft Office Excel for cleaning before finalising the same and conducting analysis in Stata version 17. Data was analysed descriptively in the form of tables and graphs through univariate tabulations and cross tabulations. Visuals for the respective tables were produced in Microsoft Office Excel.

Qualitative Data: The group discussions, in-depth interviews and the key informant interviews were recorded, transcribed, and translated into English for analysis, ensuring all responses were captured accurately, including pauses, emphasis, and non-verbal cues where relevant. The transcripts were cleaned which involved checking for errors of misheard words or typographical errors, ambiguous or incomplete responses. The main method for analysing this data was thematic analysis to identify key themes related to barriers, facilitators, and experiences of SRHR services access, education and information. Open coding was used to identify points that stood out from the discussions.

1.3.5. Ethical considerations

Informed Consent: The informed consent process prioritized participant understanding and autonomy. A clear explanation of the purpose and objectives of the study was given to all potential participants and to parents or guardians of minors. Participants were given opportunities to ask questions and receive clarification before giving their consent or assent. They were also made aware that they are free to withdraw from the study at any time, without any repercussions. The consent process thoroughly explained how the collected data would be used, ensuring participants were informed about confidentiality and anonymity protections. Participants were reassured that their decision to participate or to withdraw would not affect their employment, access to services, or eligibility for future research participation.

Confidentiality: No personally identifiable data was collected during the study. All data stored on password-protected devices was securely managed, with access restricted to the insight-gathering team.

Cultural Sensitivity: Displaced persons come from different backgrounds, vulnerabilities and the context they come from is complex. Ensuring that cultural norms are respected helps safeguard dignity

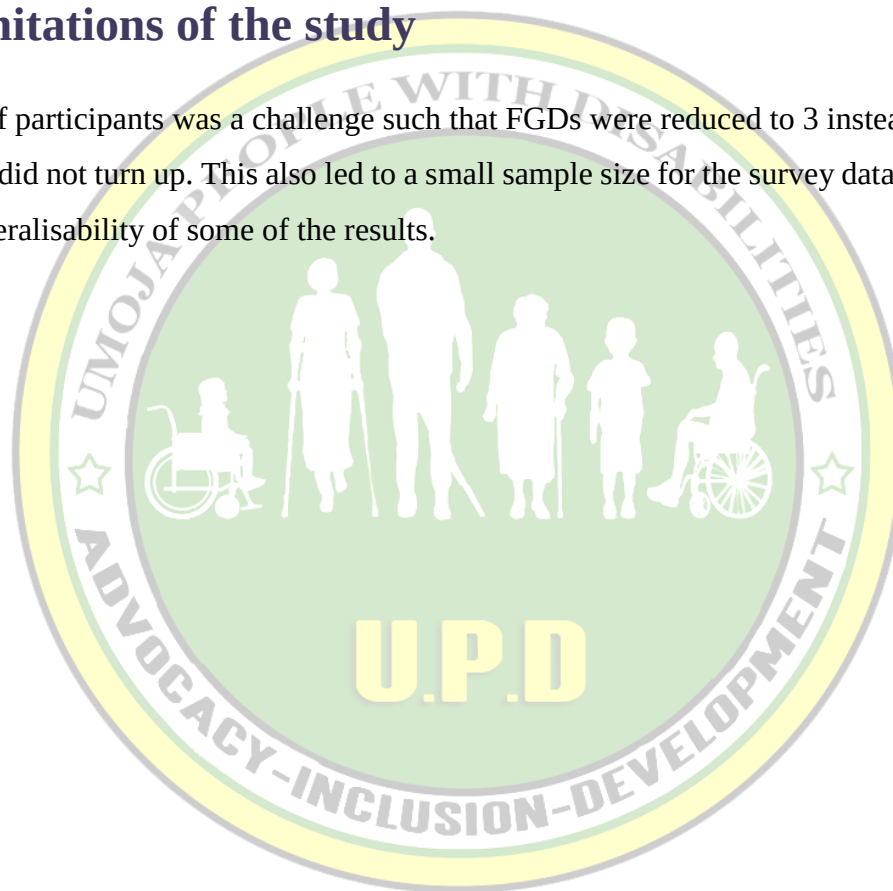
and rights. In conducting the assessment, the processes, data collection tools are going to be sensitized to the different cultures operating in the context of the camp. Measures were taken to ensure that the assessment is not viewed as intrusive, disrespectful, or offensive in participant's cultural contexts. Gender roles, family dynamics and social hierarchies were also observed in the necessary steps.

Language Barrier Management

The study employed translators to assist with translation during data collection. The study team closely worked with UPD to identify the translators.

1.3.6. Limitations of the study

Mobilization of participants was a challenge such that FGDs were reduced to 3 instead of planned four as participants did not turn up. This also led to a small sample size for the survey data collection, which affects the generalisability of some of the results.



2. Findings

This section presents the results of the study which stem from the analysis of quantitative data and narratives of women and girls with disabilities including key informants. Descriptive statistics are presented as percentages, and frequency tables to highlight needs and access barriers. Cross tabulations are used to explore relationships between disability type, age, and access to SRHR services and other variables. Based on the coding process, narrative presentations are used to report the findings from qualitative data. The results are organised around themes of knowledge of SRHR, Access of SRHR services and Violence.

Figure 1: Map of Dzaleka Camp



Sociodemographic characteristics of women and girls with disabilities at Dzaleka Camp

Disability status

The collection of disability data used the short set of questions by the Washington Group of Disability Statistics (WGDDSS)⁹, which determine the functional disability status across six dimensions of difficulty. The dimensions are seeing, hearing, mobility, self-care, communicating and remembering (cognition). The questions were asked as follows:

- Seeing (Do you have difficulty seeing, even if wearing glasses?)

⁹ https://www.washingtongroup-disability.com/fileadmin/uploads/wg/Washington_Group_Questionnaire__1_-_WG_Short_Set_on_Functioning__October_2022_.pdf

- Hearing (Do you have difficulty hearing, even if using a hearing aid?)
- Mobility (Do you have difficulty walking or climbing steps?)
- Communication (Using your usual language, do you have difficulty communicating, for example understanding or being understood?)
- Self-care (Do you have difficulty with self-care such as washing all over or dressing?)
- Cognition (Do you have difficulty remembering or concentrating?)

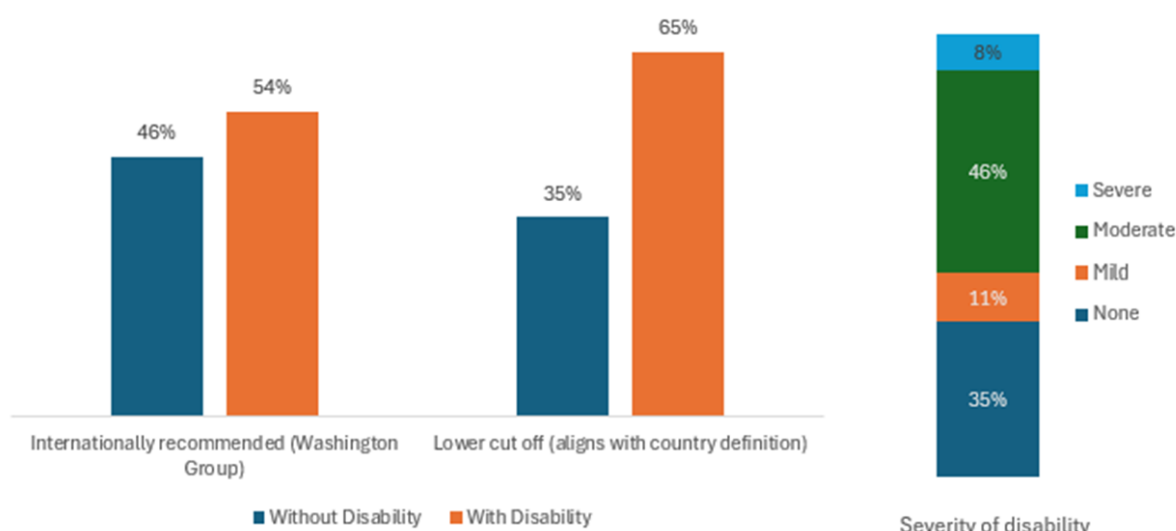
Response options to each of these questions were consistently coded as:

- No, no difficulty
- Yes, some difficulty
- Yes, a lot of difficulty
- Cannot do at all

There are three methods of deriving the prevalence of disability (disability status) from data collected using the WGDSS short set of questions above. The internationally recommended measure aggregates responses in the affirmative to the first three options (No, no difficulty, yes, some difficulty and yes, a lot of difficulty) and classifies them as having no disability. This measure yields low prevalence of disability (see 2018 Malawi census report and the thematic report on disability), as respondents with improved functionality after using assistive devices are not counted. Alternatively, a second measure classifies disability status based on responses to the first two options, and this tends to yield prevalence numbers that conform to the local definition. As such, this is the measure that was used in this study. Nevertheless, a third measure of disability status, focusing on severity, was also calculated (see Figure 2 below).

Using the first definition, the study found that 54% of the women and girls interviewed had a disability. When the second definition that usually aligns with local prevalences was used, this figure rose to 65%. Thus, we recommend quoting the latter figure for the official purposes of this study. Using the severity measure, we note that 35% of women and girls had no disability, 11% had a mild disability, 48% had a moderate disability, whereas only 8% had severe disability.

Figure 2: Disability prevalence by three definitions (Washington Group of Disability Statistics)



Background characteristics

This section outlines the demographic characteristics of women and girls with disabilities residing in Dzaleka Refugee Camp, highlighting key factors such as age, refugee status, country of origin, education, marital status, household dynamics, and economic activities. Out of the 40 women and girls sampled for this study, 37 fully responded to the questions on disability status. Table 1 summarises their socio-demographic characteristics.

The age distribution among the sampled women and girls with disabilities indicates an increasing prevalence of disabilities in older age groups. Among those aged 15-24 years, 58% have a disability, while 50% of women aged 25-34 and 35-44 years also report disabilities. Strikingly, all women in the 45-54 and 55-64 age categories have disabilities, showcasing a concerning trend that underlines the vulnerability of older demographics within this group.

In terms of refugee status, a significant portion of the sampled population consists of refugees or asylum seekers. About 20% of the surveyed population were asylum seekers, out of which 63% had a disability. Among refugees, who comprised 65% of the sample, 69% of them also had a disability of some sort. This status can significantly impact access to services and support among women and girls in Dzaleka, making it crucial to understand the environment in which they live.

On the question of country of origin, 34 participants fully responded to the question. A significant population is from Democratic Republic of Congo with 65%, followed by Burundi with 13% and Rwanda with 7%. About 15 % did not respond to the question.

The country of origin reveals notable variances in disability rates among the sampled women and girls. Of those surveyed who had identified with disabilities, 60% hail from Burundi, 69% from Congo, and 67% from Rwanda. This demographic insight highlights the need to consider the cultural and socio-political contexts of these countries when developing support initiatives.

Educational attainment is alarmingly low among this group, with many not attending school. Among those sampled, 70% are not in school, while only 43% are actively pursuing education. The highest levels of schooling completed are also concerning: 50% reported completing primary and secondary education. These figures illuminate the educational barriers faced by women and girls with disabilities and signal an urgent need for interventions.

Marital status data reflects a range of life experiences faced by this population. A notable 78% are currently married, while all (100%) of the divorced individuals reported having a disability. Additionally, 56% of respondents identified as never married, 75% as separated and 50% as widowed. These marital dynamics suggest that societal and cultural factors may play a role in the lives of women and girls with disabilities in the camp.

Household dynamics indicate that many women and girls with disabilities are heads of their households. Of the sampled population that were household heads, 69% had a disability, whereas 67% those not heading houses had a disability. This data points to the significant responsibilities shouldered by these individuals, often without adequate support.

Access to economic activities is vital for the empowerment and sustainability of women and girls with disabilities. The study shows that 59% of respondents do not have access to economic opportunities, highlighting the barriers they face in achieving financial independence. Conversely, 80% report having some level of access to economic activities, suggesting the potential for targeted support programs to enhance their participation in this area.

In conclusion, the demographic characteristics of women and girls with disabilities at Dzaleka Refugee Camp reveal significant challenges, including barriers to education and economic participation. By purposefully sampling this vulnerable population, the study underscores the need for targeted interventions aimed at improving their living conditions and opportunities for empowerment. Understanding these demographics is crucial for developing support mechanisms that can meaningfully impact their lives.

Table 1. Distribution of sampled women by background characteristics and disability status

	Disability status			
	Without Disability	With Disability	Total	Obs
	%	%	%	
Age group				
15-24 Years	42	58	100	12
25-34 Years	31	69	100	13
35-44 Years	50	50	100	6
45-54 Years	20	80	100	5
55-64 Years	0	100	100	1
Refugee status				
Asylum seeker	38	63	100	8
Refugee	31	69	100	26
Country of Origin				
Burundi	40	60	100	5
Congo	31	69	100	26
Rwanda	33	67	100	3
School attendance				
Not in school	30	70	100	30
In school	57	43	100	7

Highest level of schooling?				
Completed primary education	50	50	100	2
Completed secondary education	50	50	100	2
Never attended school	67	33	100	3
Some primary education	18	82	100	11
Some secondary education	33	67	100	15
Tertiary education	50	50	100	2
Marital status				
Currently married	22	78	100	9
Divorced	0	100	100	2
Never married	44	56	100	16
Separated	25	75	100	4
Widowed	50	50	100	6
Household headship				
Not head	33	67	100	21
Head of house	31	69	100	13
Do you have access to economic activities?				
No	41	59	100	27
Yes	20	80	100	10
Total	35	65	100	37

2.1. Knowledge of Sexual and Reproductive Health and Rights Services

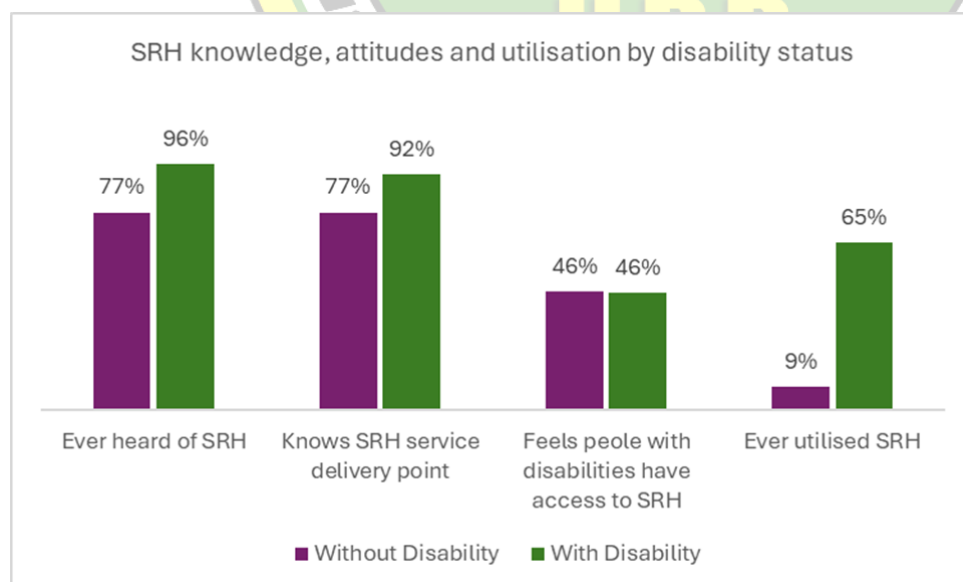
Findings reveal that women with disabilities had higher knowledge of Sexual and Reproductive Health and Rights services. However, younger women and girls demonstrated a lack of knowledge of the service delivery points where they could access the services.

2.1.1. High knowledge, attitudes of SRHR among women and girls with disabilities

Figure 3 highlights the sexual and reproductive health and rights (SRHR) knowledge, attitudes, and practices of women and girls at Dzaleka Refugee Camp, comparing those with and without disabilities. Notably, 96% of those with disabilities have ever heard of SRHR compared to 77% of those without disabilities. Furthermore, while 92% of women and girls with disabilities know where to access SRH services, only 77% of those without disabilities are aware of service delivery points.

In terms of perceptions regarding accessibility, both groups show the same level of belief—46%—that individuals with disabilities have access to SRHR services.

Figure 3: sexual reproductive health (SRH) knowledge, attitudes and utilisation



Generally high levels of SRHR knowledge within the camp, which can inform health and education interventions.

Figure 4 illustrates the level of knowledge of SRHR across various background characteristics. Notably, all respondents aged 45-54 years and between 55-64 years reported having heard of SRHR, indicating a complete awareness in these age groups. Similarly, 100% of those who are currently married and never married also expressed awareness of SRHR. Other demographics show slightly lower, yet substantial awareness. For instance, 92% of women and girls aged 15-24 years and 85% aged 25-34 years have heard of SRHR.

Among educational categories, 100% of those currently in school reported awareness, while 88% of those who are out of school also indicated the same. Additionally, 93% of economically active individuals reported being aware of SRHR, compared to 80% of those who are economically inactive. The high level of knowledge amongst respondents aged 45 to 55 could be attributed prenatal and postnatal education that happens at health facilities to all child bearing women as observed in the narratives of women below :

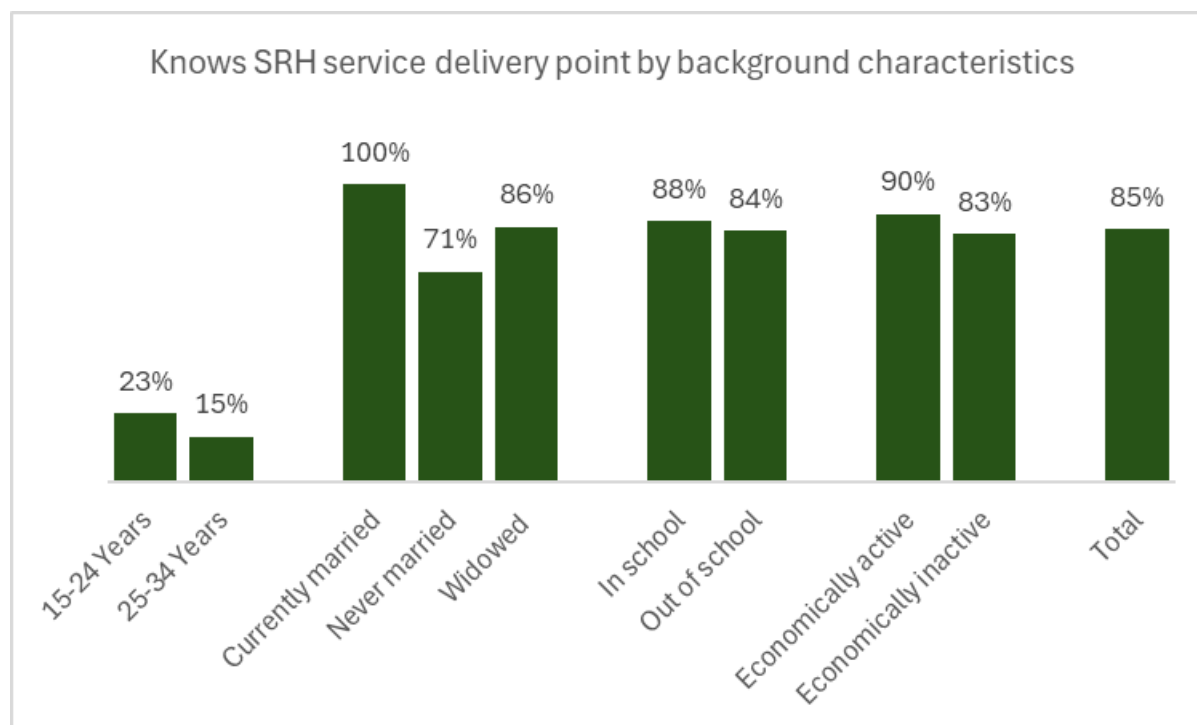
Most of the information I know is from the chipatala (health centre). When you are pregnant, there are always classes that teach you about family planning, prevention of HIV from mother to child but also testing of blood. So i can say that the chipatala really helped in my case - Interview with a woman with disability.

Low knowledge of SRH service delivery points among younger women.

In Figure 5 below, we note considerable variation across different background characteristics. Notably, all of the currently married individuals reported knowing where to access SRH services, indicating a high level of awareness among this group. Additionally, 86% of widowed respondents and 88% of those currently in school also know their service delivery points.

In contrast, awareness is considerably lower among younger age groups; only 23% of women and girls aged 15-24 years are aware of SRH service delivery points, and just 15% of those aged 25-34 years report the same. Among those who are economically active, 90% know where to access SRH services, while 83% of economically inactive individuals share this knowledge. The overall awareness rate across all respondents stands at 85%. These findings highlight significant gaps in knowledge, particularly among younger women and girls, emphasizing the need for targeted educational initiatives to enhance awareness of available SRH services within the camp. Furthermore, it would be advisable for future SRHS in the camp to target the age bracket of girls and young women with disabilities.

Figure 4 : Level of knowledge of SRH across various background characteristics.



2.1.2. Sources of SRHR Information: Peers and Friend are a significant source of information

Findings reveal that friends and peers are the most significant source of information, with 40% of respondents citing them as a key reference point. This highlights the importance of social circles in shaping awareness and understanding of SRHR issues. Community health workers and volunteers also serve as essential resources, with 33% of respondents relying on them for information. In addition, community members, including family and acquaintances, account for 23% of SRH information sources, reflecting localized support systems. Media channels, particularly radio, are utilized by 20% of participants, indicating the impact of broadcast media in reaching audiences with crucial health messaging.

However, a notable 10% of respondents either declined to respond or provided no answer regarding their sources of information. Other sources, such as the healthcare delivery system, youth clubs, school social groups, local media, and parents, each account for smaller percentages, ranging from 5% to 10%.

These findings emphasize the need for enhancing outreach efforts that build on peer networks and community health initiatives to improve SRHR knowledge and resources within the camp. It would

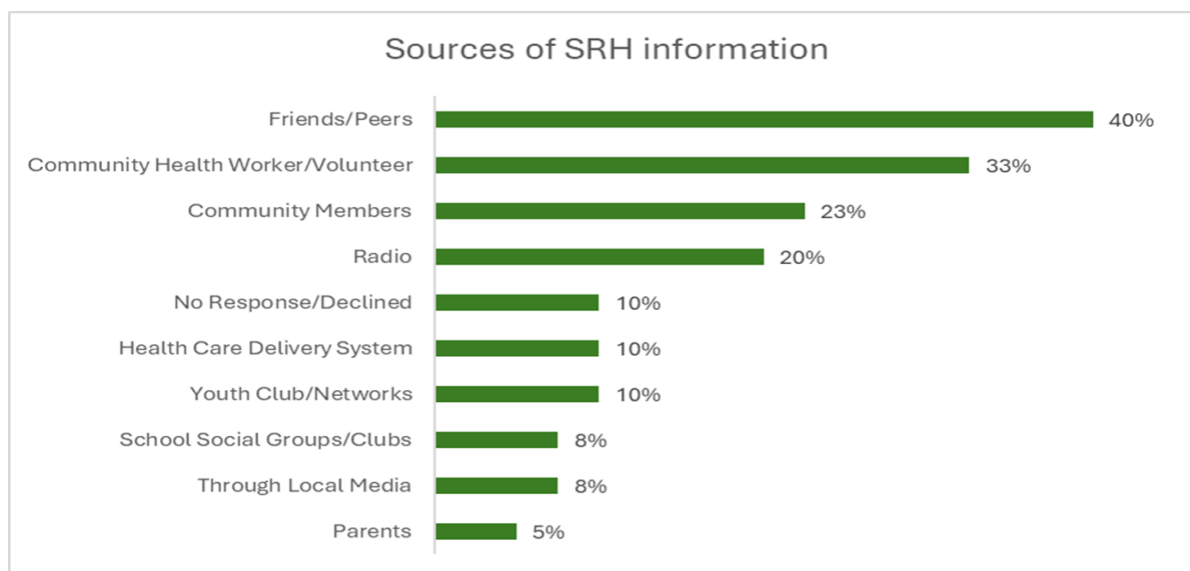
therefore be important for local Non- Governmental organisations to develop interventions on community outreach services and empower local volunteers on sexual reproductive health services. Community outreach interventions are particularly important for girls with disabilities whose access to SRHS is also impeded by negative societal perceptions. Interactions with study respondents further revealed that local NGOs did not have programs on sexual reproductive health despite the opportunity of working closely with women with disabilities. This is evidenced below,

They (NGO's program's office) used to tell us, just go and see, if there are problems, they must tell you what they are facing when coming. So, this teaching or educating them about family planning or why are you still having babies, it's like we never talked about. Women Leaders, local NGO.



Sometimes we get information at Dzaleka health center , they have a place there where they talk about these things (SRHR) but it is not easy to go there because of my mobility challenges but information that finds me at home or within the community is easy to access- Girl with disability.

Figure 6: Distribution of sources of SRH information

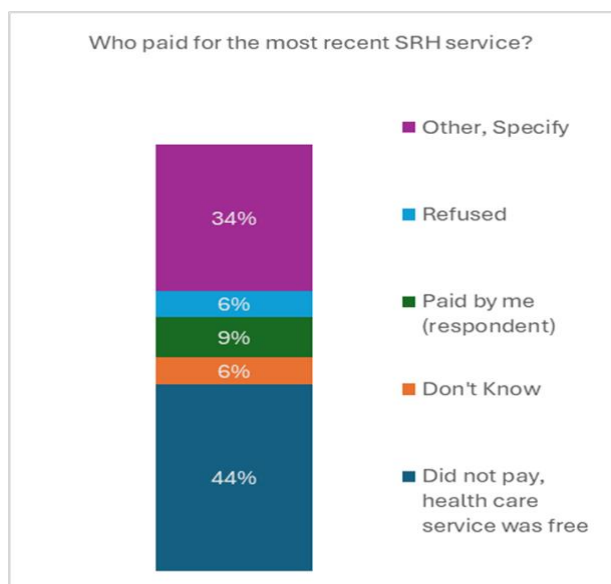


2.2. Access to Sexual and Reproductive Health and Rights Services

Majority of SRHR services is accessed for free

When asked who paid for the SRH service accessed, a significant portion, 44%, did not pay for the service, as it was provided free of charge. Additionally, 34% of respondents selected "Other" when asked about payment sources, indicating a need for further clarification on this category. Only 9% reported paying for the service themselves, while 6% indicated that they did n't know who paid. Another 6% refused to answer this question. These findings signify the importance of affordable or free SRHR services in communities of economically vulnerable populations such as camps, more to women and girls with disability (see Figure 7).

Figure 7: Financing of individual SRH services



There is privacy and supportive interactions with SRHR workers

A notable 50% of respondents felt that there was privacy during their visit, indicating a positive perception of the confidentiality maintained in these services. Additionally, 47% reported receiving enough information and services, which suggests a generally satisfactory level of communication and care provided. Moreover, an equal 47% of respondents noted that health providers were friendly, contributing to a supportive atmosphere during their visits. In contrast, 18% indicated that they were referred to a different facility, which may reflect gaps in service provision at the initial point of care (See Figure 13). The privacy and supportive interaction with SRH workers especially at the health centre is due to deliberate efforts which the institution has implemented because of a health worker who volunteers at UPD.

*I am a health worker who also volunteers at a community based organisation that works with people with disabilities. I also work as one of the leaders at the single mother's association here at the camp. I use that experience to help the health centre to be inclusive, especially on SRH Issues. **Health worker, local health centre.***



At service delivery points, people with disabilities are assisted quicker than those without disabilities

Participants were also asked how long they waited before being assisted. The data regarding assistance times for persons with disabilities (PWD) compared to those without reveals that PWD are often assisted more quickly, particularly within the 5 to 30-minute timeframe. However, individuals without disabilities show longer wait times, especially in the categories exceeding 60 minutes. For the 45 to 60-minute range, assistance times are somewhat more evenly distributed, yet PWD still receives slightly quicker service.

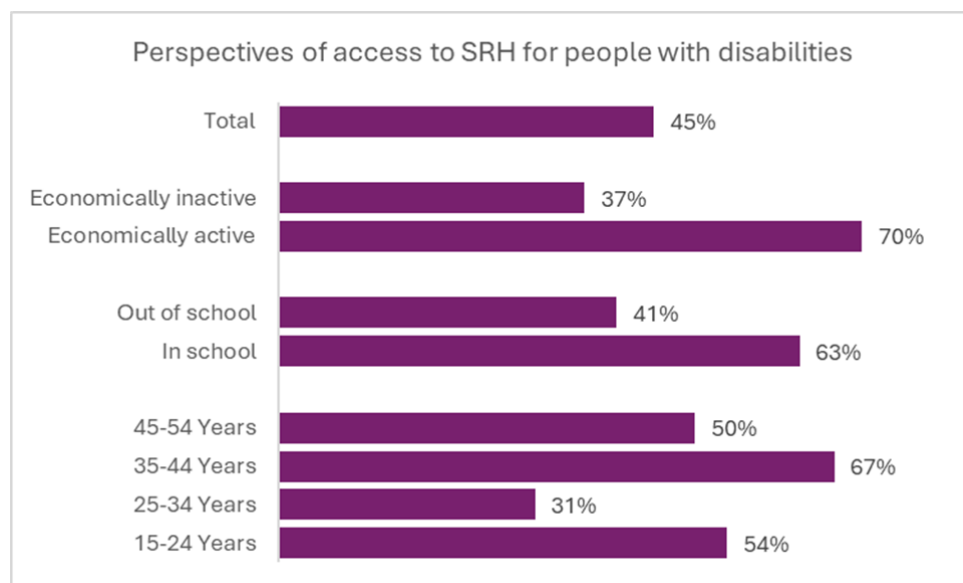
At Chipatala , we get quick service there once they note that you have a disability. The only challenge is that they only respond quickly when one has a physical disability that is identifiable. Those of us with other disabilities which they cannot see, it is difficult to get help - **Woman Leader, Local NGO**

Social demographic characteristics like age, economic status and education influence perspectives on whether people with disabilities can access SRH services.

There are varying perspectives on access to sexual and reproductive health (SRH) services for people with disabilities among women and girls at Dzaleka Refugee Camp. Overall, 45% of respondents believe that individuals with disabilities have access to SRH services. Notably, those who are economically active expressed higher confidence in access, with 70% affirming availability, compared to only 37% of economically inactive individuals.

When considering educational status, 63% of respondents currently in school perceive that people with disabilities can access SRH services, while this belief drops to 41% among those out of school. Age also influences perspectives: 67% of respondents aged 35-44 years and 54% of those aged 15-24 years believe access is available, contrasting with just 31% of the 25-34 age group and 50% of those aged 45-54 years. Figure 8 demonstrates these findings.

Figure 8: Perspectives of access to SRH for people with disabilities



A disproportionately high percentage of economically active, out of school, and currently married women and girls have ever used SRHR services.

When asked whether they had ever used any SRHR service, a total usage rate of 46% was reported among women and girls at Dzaleka Refugee Camp. Notably, those who are economically active exhibit a significantly higher usage rate of 70% compared to just 37% among the economically inactive group. Educational status also plays a crucial role; 53% of respondents in school report having used SRHR services, while only 14% of those out of school have accessed these resources.

Marital status further influences usage patterns, with a striking 78% of currently married individuals reporting ever use of SRHR services, compared to only 20% of never married respondents. Age demographics indicate that 62% of those aged 25-34 years and 60% of those aged 45-54 years have utilized SRH services, while 50% of respondents aged 55-64 years also report usage. In stark contrast, only 18% of individuals aged 15-24 years have ever used SRH services. These findings highlight significant disparities in access and utilization of SRH services (see Figure 9).

Figure 9: Ever use of SRH by background characteristics

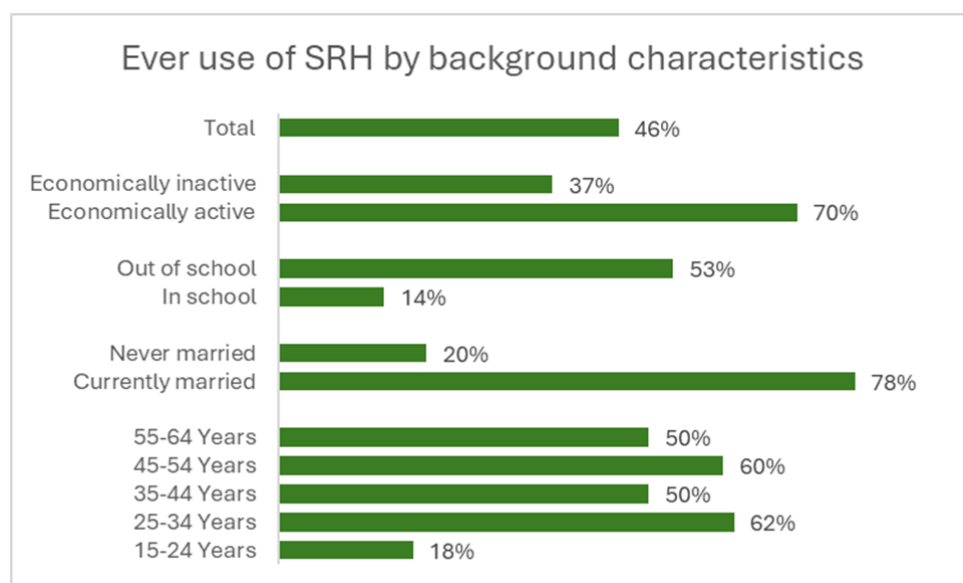


Table 2 illustrates the ever use of SRH, services received at the last visit and services needed but not provided by type of service and disability status. Notably, utilisation of sexual and reproductive health (SRH) services highlights significant disparities based on disability status. Overall, services such as abortion/post-abortion care and reproductive cancer treatment are exclusively accessed by individuals with disabilities, showing a complete reliance on these services among that group. In contrast, individuals without disabilities show minimal usage, as evidenced by the 0% for both services.

Table 2: Ever use, last use and accessibility of SRHR by service

Service	Ever utilised		Received at last visit		Needed but not provided
	Without disability	With disability	Without disability	With disability	
Abortion/post-abortion care	0%	100%	0%	100%	0%
Reproductive cancer treatment and care	0%	100%	0%	100%	4%
HIV Testing and Counselling	10%	90%	10%	90%	4%

Contraceptive counselling	17%	83%	33%	67%	4%
Contraceptive methods	14%	86%	9%	91%	7%
Fertility/Infertility care			0%	100%	4%
Referral to health facility/other SDP					4%
Prevention, diagnosis and management of STIs	100%	0%	0%	100%	0%
Ante-natal, delivery and post-natal care services	0%	100%	0%	100%	0%
Prevention of Mother to Child Transmission of HIV (PMTCT)	0%	100%	0%	100%	0%
Treatment of sexual abuse (Including PEP)	100%	0%			0%
Obtain ART/ARVs	0%	100%			0%
Menstrual hygiene management	25%	75%			0%
Sexual health education	0%	100%	100%	0%	0%
Did not receive any service	100%	0%	80%	20%	14%
No Response/Declined	60%	40%	50%	50%	64%
Other (Specify)	0%	100%			11%

Most accessible SRHR services

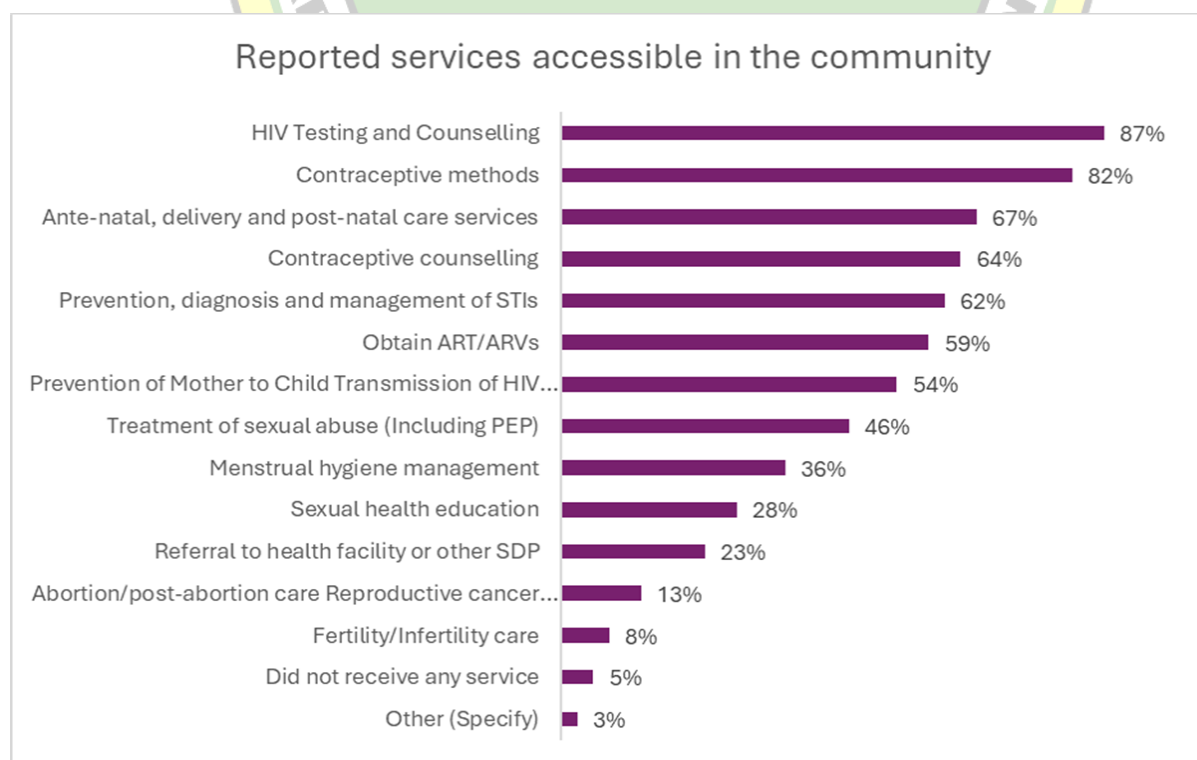
HIV testing and counselling and contraceptive methods are the most accessible SRHR services

Respondents were asked to mention the most accessible services within their community, as outlined in Figure 10. HIV testing and counselling stands out as the most commonly accessible service, with an impressive 87% of respondents reporting availability. Following closely is the availability of contraceptive methods, noted by 82% of participants, underscoring the importance of family planning resources in the community.

Other significant services include ante-natal, delivery, and post-natal care, reported accessible by 67% of respondents, along with contraceptive counseling at 64%. Moreover, the prevention, diagnosis, and management of sexually transmitted infections (STIs) is cited by 62%, and access to antiretroviral therapy (ART) and ARVs is acknowledged by 59% of individuals.

However, there are areas indicating lower levels of service access. For instance, menstrual hygiene management is acknowledged by 36%, and sexual health education is reported by 28%. Referral to health facilities or other service delivery points (SDPs) is available for 23%, while access to abortion or post-abortion care is noted by 13%. A smaller percentage of individuals report fertility/infertility care at 8%, and 5% stated that they did not receive any services at all. These findings illustrate the need for continuous efforts to enhance the availability and accessibility of comprehensive SRHR services, particularly focusing on areas with lower reported access.

Figure 10: Accessible services in the community



HIV testing and counselling indicate a higher engagement among individuals with disabilities at 90%, compared to just 10% for those without disabilities. Contraceptive methods and counselling also display similar trends, with 86% and 83% usage respectively among women with disabilities, whereas women without disabilities reported significantly lower figures of 14% and 17%. Additionally, some services, like the prevention, diagnosis, and management of STIs, are only reported by individuals without disabilities at 100%, illustrating a substantial gap in access. Moreover, a significant portion of respondents noted services they needed were not provided, particularly in categories such as adolescent health education and menstrual hygiene management. These findings underscore the urgent need to improve access and address the service gaps for both individuals with and without disabilities in the camp.

Barriers to accessing SRHR services

Several factors were noted to hinder accessibility of SRHR services in the camp. General scepticism regarding effectiveness of SRHR, negative attitudes of health personnel, long distances were noted as key factors.

General scepticism regarding the effectiveness of SRHR services in the camp

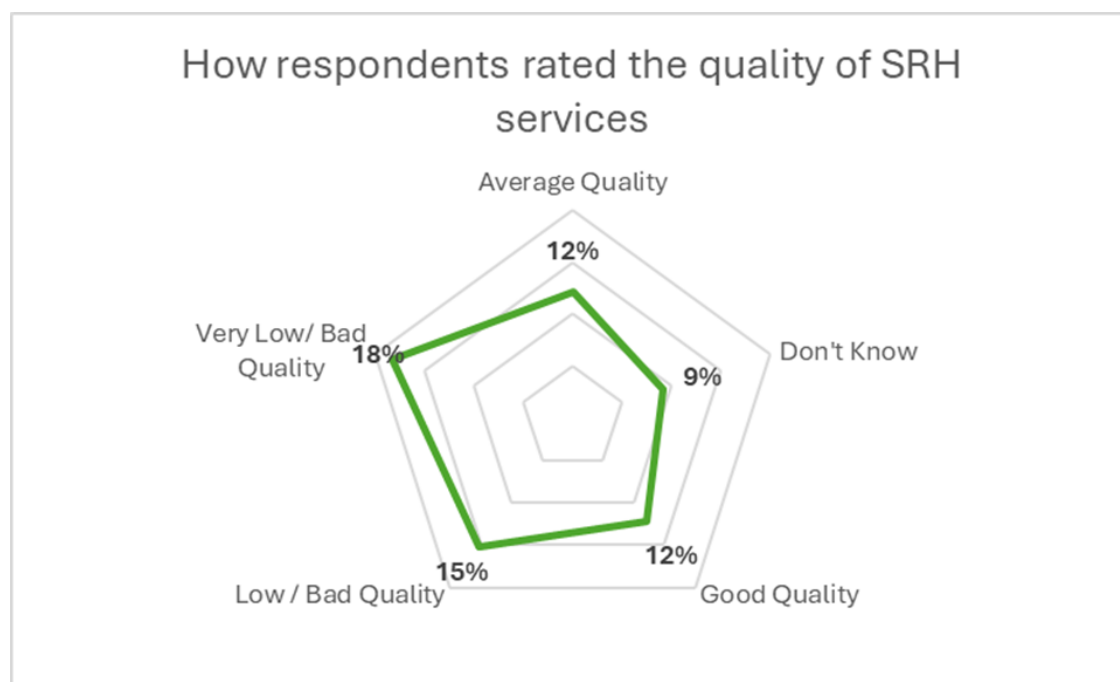
Women and girls at the camp were asked to rate the quality of SRHR. Results indicate mixed perceptions. A noteworthy 18% described the quality of services as "very low/bad," indicating significant concerns about the care received. Additionally, 15% rated the services as "low/bad quality," further highlighting areas for improvement. Some of the reasons cited for the above perceptions included past negative experiences which included complications after accessing the services. For example, some of the respondents shared the following sentiments;

Like me, before the time I was giving birth, I was using a loop, but once I used tablets (contraceptive pills) eish, it almost killed me.' **KII Women Leader, local NGO.**

In contrast, a smaller proportion—12% rated the services as average quality, while another 12% felt the quality was good. Moreover, 9% of respondents were unsure or did not know how to classify the service quality. This distribution suggests a general skepticism regarding the effectiveness of SRHR services in the camp, emphasizing the importance of enhancing service delivery to improve user

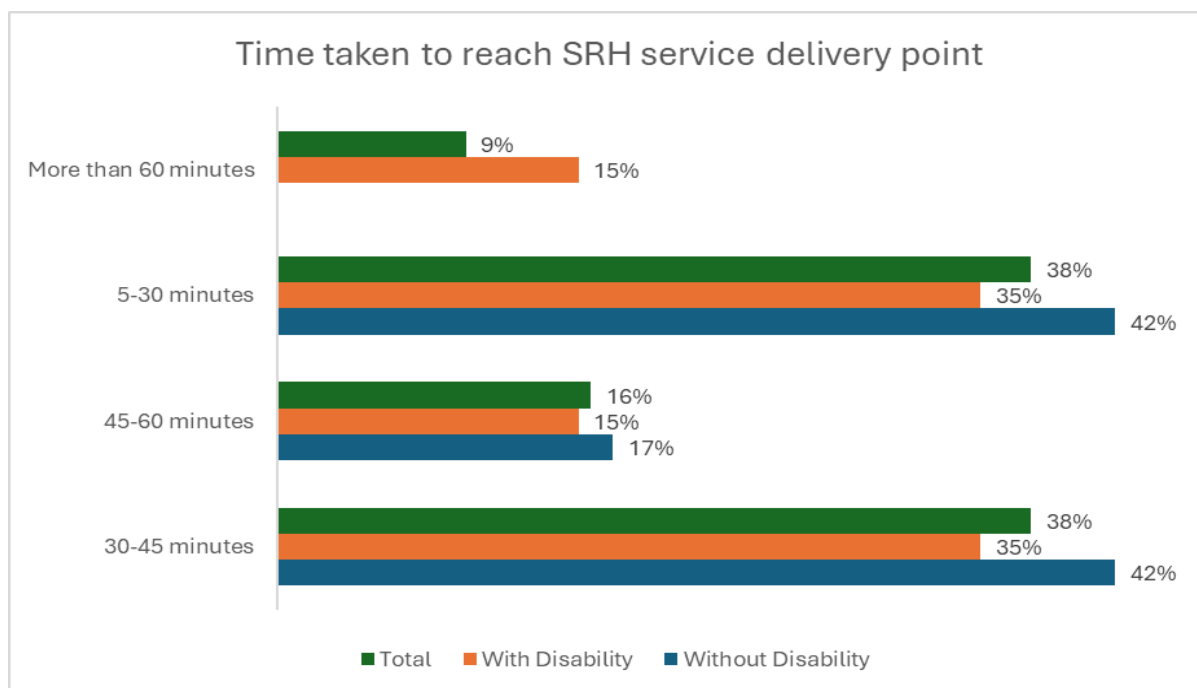
satisfaction and overall quality of care. Addressing these concerns will be crucial for fostering trust and encouraging greater utilization of available SRHR resources.

Figure 11: Respondents rating of SRHR services



About 15% of PWD takes over an hour to reach SRHR service delivery point

When asked how long it took participants to reach an SRHR service delivery point, notable differences were revealed between respondents with and without disabilities. For the majority, 38% of those surveyed reported that it takes between 5 to 30 minutes to reach the services, with 42% for those without disabilities indicating similar travel times. Regarding longer durations, 16% of participants indicated it takes 45 to 60 minutes, with a slightly higher percentage (17%) for those without disabilities. Meanwhile, 9% reported travel times of more than 60 minutes, which rises to 15% among those with disabilities—a significant consideration for accessibility. Similarly, 38% of respondents stated it takes 30 to 45 minutes, with 35% from the disabled group reporting the same. This highlights the need for improved service accessibility and potentially enhancing the proximity of resources to better serve the community's needs.



Stigma and discomfort play a major role in hindering access to SRHR services.

Participants who indicated having never utilised SRHR were asked to provide a reason for not doing so (see Figure 12). The most prevalent reason, cited by 52% of respondents, is feeling shy, indicating that social stigma and discomfort play major roles in hindering access to SRHR services.

Additionally, 19% of participants reported they never had the need to access these services, suggesting a potential lack of awareness or understanding of their SRHR needs. Other reasons include 15% who are unaware of available SRHR options, and 4% who mentioned previously having had bad experiences with healthcare services. An equal 4% stated that their partners disapprove of accessing SRHR services, and another 4% indicated they had never thought about seeking such services.

Figure 12: Reasons for not using SRHR

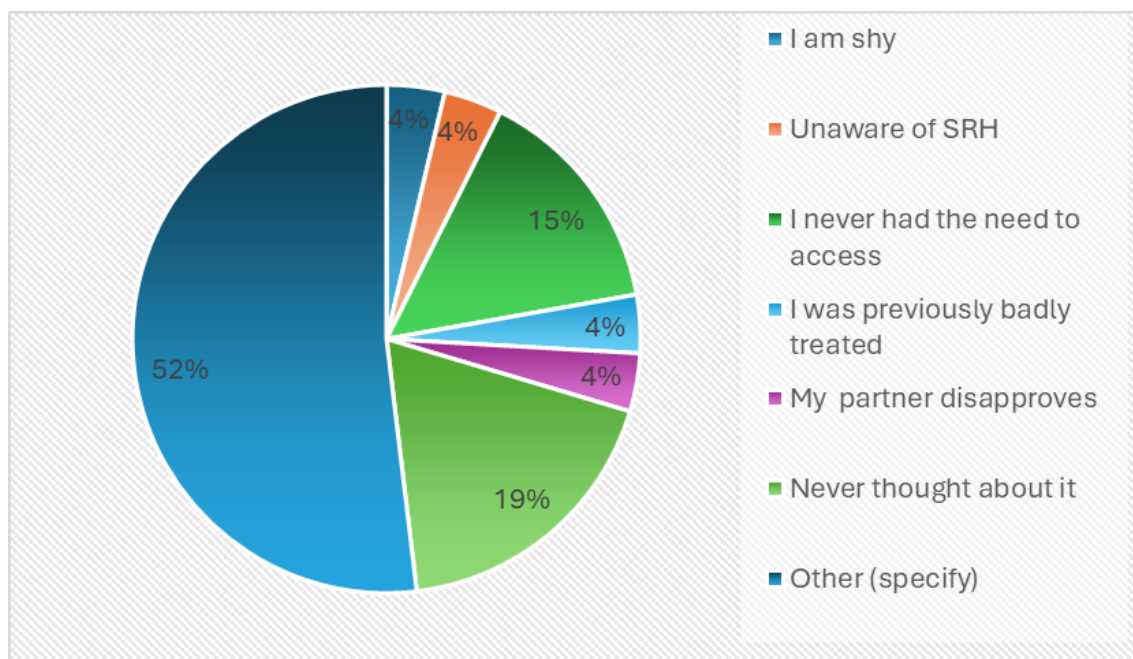
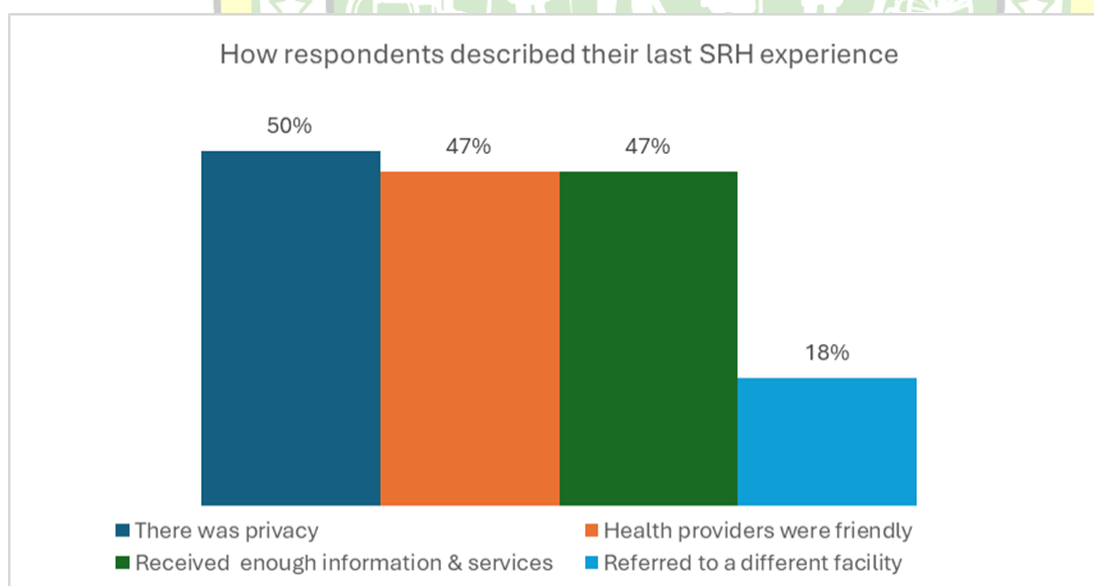


Figure 13: Respondent's description of their SRHR service use experience.



Access to SRHR services is also hindered by long distances (32%), inaccessibility by PWD (32%) and unfriendly attitudes of health workers (29%)

Findings on the challenges faced when accessing SRHR services highlight several significant barriers, with long distances cited as the most prevalent issue, affecting 32% of respondents. This is closely followed by concerns regarding unfriendly attitudes of health workers, which 29% of respondents also noted as a barrier. During interactions with key informants, they explained that health personnel tended to antagonize women with disabilities when accessing SRHR services particularly maternity services as they are deemed not having the capacity to bear children. As such, women with disabilities tend to shy away from accessing antenatal services and only seek support when they are advanced in their pregnancy. One of the women leaders had this to say;

they (women with disabilities) don't go to the hospital because when they go, they always blame them saying why did you get pregnant. You know what the situation is like and they don't give them a chance to deliver like other people. They just write a letter to go to the Bottom (Hospital) for a C-section.

Women Leader, Local NGO.

Other challenges include the inaccessibility of services for people with disabilities (32%) coupled with a shortage of health workers (18%).

you know this health centre and when you tell them I want this method; they will say okay. They can't even give you advice, like saying, you know, you're too big and this is not going to work for you and that's why you will find that many don't want to use. They don't want to use family planning because they'll tell you; no, if I use this, I will always bleed. They (specialists) don't explain; if this one doesn't work, come, we will give you this. **Women Leader, Local NGO.**

Cultural or religious beliefs posed issues for 12% of respondents. Interactions with adolescent girls revealed a shared view that contraceptives would make them barren and that they would have difficulties when giving birth.

The way people speak about family planning methods is that they are bad for our health. For instance, they said when you use a family planning method, you cannot give birth in the future and teenage pregnancies are coming because of this challenge because when you want to have access to these family planning methods, you find that your own family cannot allow it. So, instead, you get a pregnancy without actually planning for it. **Girl with disability, FGD.**

Similar sentiments were echoed by other respondents,

Some people usually say that those methods are not good for girls because they may close their womb and they can't have another child. My stomach was paining a lot on this side (pointing to the side) and

*that's why I decided to stop and that is also the reason that many people with disabilities are not going to access these family planning services. **Girl with disability, FGD.***

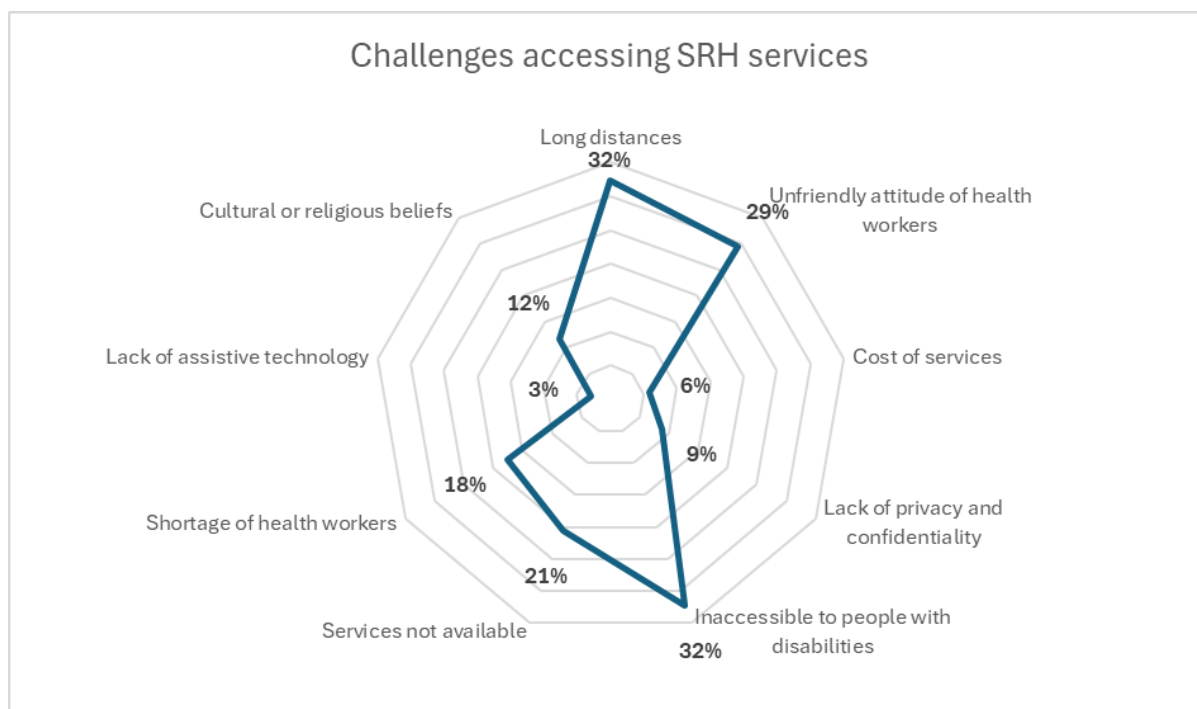
Other prevailing beliefs included that much in camp so much that some family planning methods like condoms may bring infections to girls and destroy their body. This was also captured from the interactions with adolescent girls.

*... those oils from the condoms are bringing infections to many girls. So, if you're going to use condoms daily, you're going to suffer from infections. **Girls with disability, FGD.***



As such, girls indicated that they prefer using the natural methods of family planning, particularly observing the calendar days. Much as this could be effective for pregnancy prevention, the method carries a risk of transmission of sexual infections. This points to the need for sex education campaigns so that adolescent girls are reached with the right information.

Notwithstanding that, 21% reported that services were simply not available to them. Additionally, 6% mentioned the cost of services as a barrier, and 9% highlighted a lack of privacy and confidentiality.



Overall, these findings underscore multiple systemic issues that complicate access to SRHR services, emphasizing the need for targeted improvements in service delivery to better meet community needs.

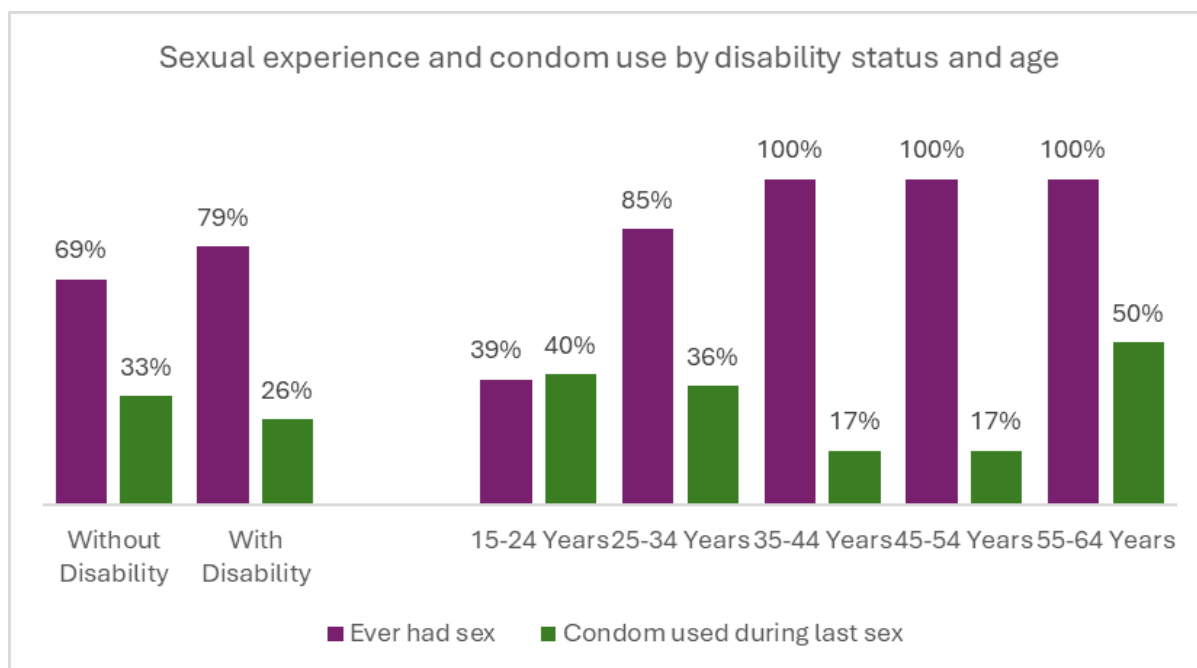
2.3. Sexual experience, contraception and menstrual health and hygiene management

2.3.1. Sexual Experience

High sexual activity despite inconsistency in condom use

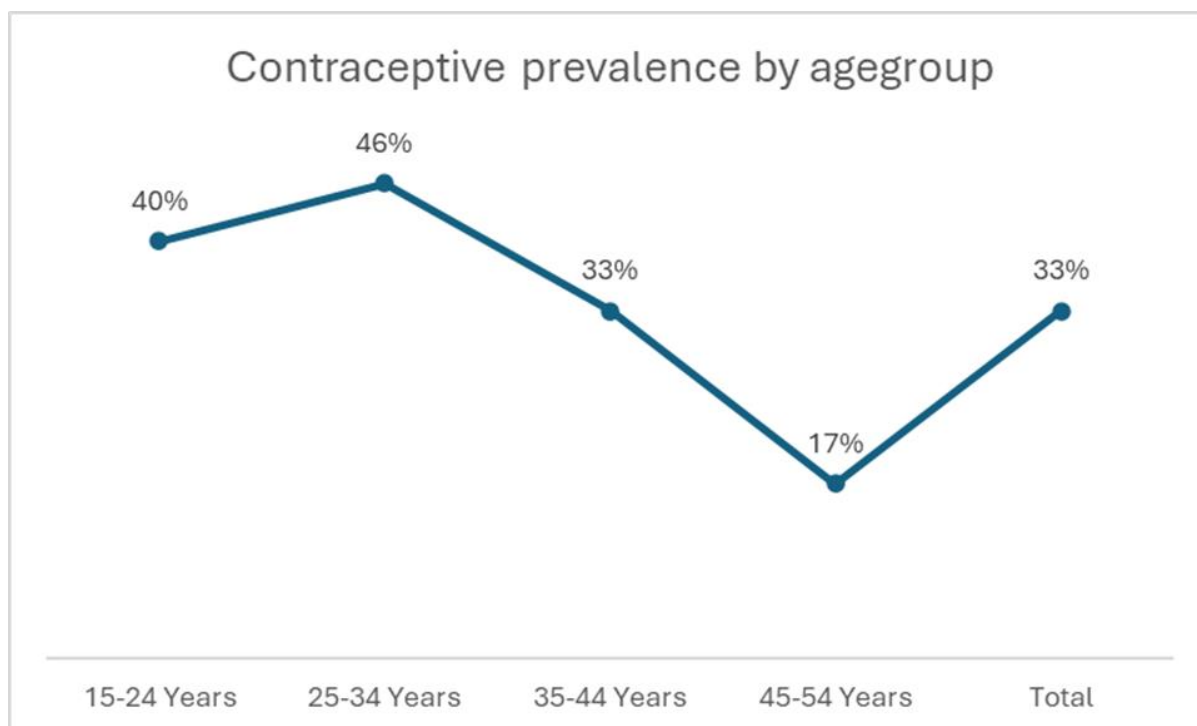
Findings on sexual experience and condom use highlight notable differences between individuals with and without disabilities across various age groups. Among those without disabilities, 69% reported having ever had sex, compared to 79% of individuals with disabilities. However, condom use during the last sexual encounter shows disparities, with only 33% of those without disabilities using condoms, versus 26% for those with disabilities. Age-wise, the highest rates of sexual experience were reported among older individuals. Condom use among these age groups varied, with a notable 40% usage among the 15 to 24 and 36% for the 25 to 34 age groups. It was the highest (50%) among those aged 55 to 64.

Figure:



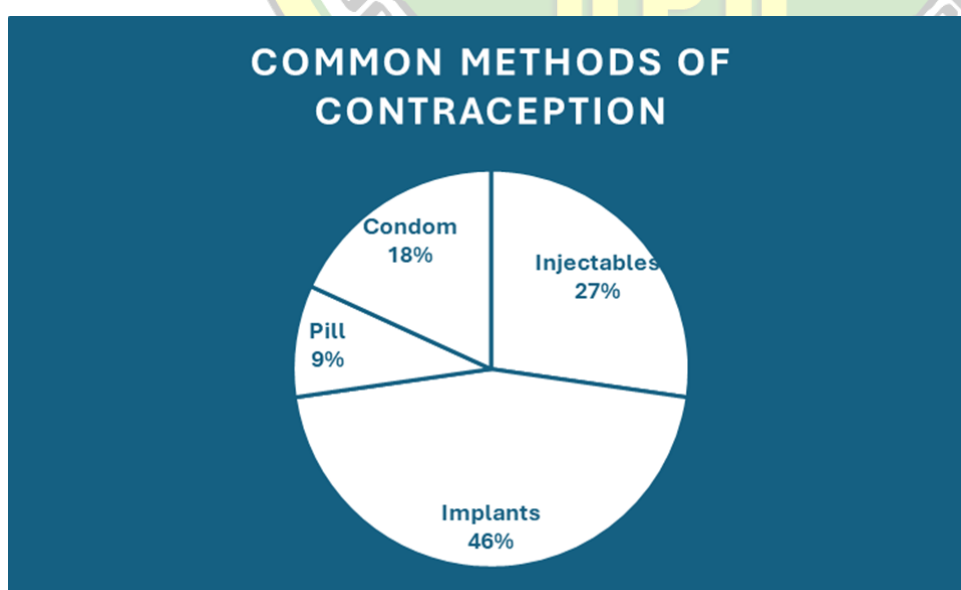
2.3.2. Contraception

Findings on contraceptive use by age group reveal significant variations in usage among different demographics. In the 15-24 age group, 40% of individuals reported using contraception, while usage increases to 46% among those aged 25-34. However, there is a noticeable decline in prevalence among older age groups, with only 33% of individuals aged 35-44 utilizing contraception and a mere 17% in the 45-54 age group. This trend appears to stabilize again for those aged 55. Overall, contraceptive prevalence rate was reported at 33%.



Common methods of contraception

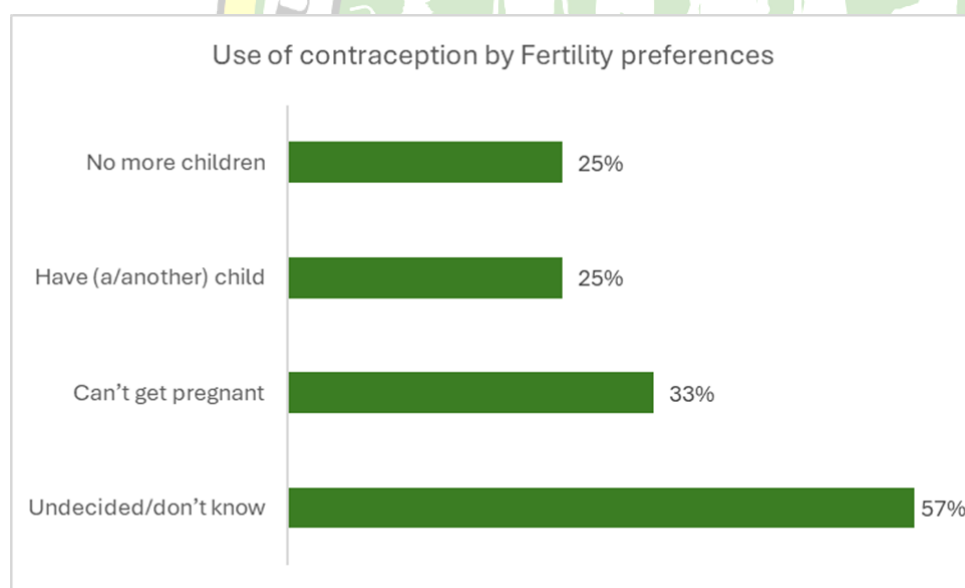
Looking at the types of contraceptive methods used, implants topped the list at 46% followed by Injectables at 27% and condoms at 18%. Pills were the least common method among women and girls at Dzaleka Camp.



75% of women don't want any more children, but are not using family planning methods

The data on contraceptive use by fertility preferences indicates the percentage of women using contraception within different reproductive intentions. Among those who do not want any more children, only 25% are currently using contraception, suggesting a 75% that isn't using. Similarly, for women who wish to have another child, 25% reported using contraceptives. Surprisingly, a notable 33% of women who cannot get pregnant are still using contraception, possibly due to personal beliefs, medical advice, limited knowledge or a desire to avoid unintended pregnancies during a time when they are trying to conceive. Lastly, a significant 57% of individuals who are undecided or do not know about their fertility preferences are also using contraception.

This data highlights that contraceptive use varies significantly across different fertility intentions, with many women not using contraception even when they have clear desires regarding childbearing. It emphasizes the need for targeted education and support that addresses individual fertility goals and contraceptive methods.



Among the women and girls not using any contraception at Dzaleka Camp, the data on reasons for not using contraception highlights several key factors influencing this decision among individuals. A significant 44% of respondents indicated that they do not currently have a sexual partner, which is the primary reason for abstaining from contraceptive use. Another 13% reported that they are abstaining from sex altogether, suggesting a conscious choice to refrain from sexual activity. Additionally, 6% of individuals stated they want (more) children, indicating their intention to conceive. Another 6%

mentioned that they believe contraception reduces sexual satisfaction, which could influence their preferences regarding reproductive health.

Overall, these findings reveal a dominant theme of relationship status affecting contraceptive practices, and they underscore the importance of targeted education and resources to address diverse reasons for not using contraception.

2.3.2. Menstrual health and hygiene

Respondents with the most recent menstrual period reported relatively more positive menstrual hygiene management practices

Findings on menstrual hygiene management provide insight into the experiences of individuals based on their last menstrual period. Among those who responded, 53% reported their last menstrual period was less than two weeks ago, with 29% indicating they were unable to attend social activities during this time. In contrast, 31% noted their last period was within a month, with 20% unable to participate in social activities. Remarkably, 94% of those whose last period was less than two weeks ago stated they were able to wash and change in privacy, compared to 80% of those within a month.

As for menstrual hygiene products, 100% of the respondents in the most recent period used wool, tampons, cups, or cloth, while the reusability of materials was reported at 53% overall.

Last menstrual period	Proportion responded	Was unable to attend social activities	Was able to wash & change in privacy	Used wool, tampons, cup or cloth	Materials were re-usable
Less than 2 weeks	53%	29%	94%	100%	53%
Within a month	31%	20%	80%	90%	40%
One to three months ago	3%	100%	100%	100%	0%

Over three months ago	13%	50%	75%	100%	25%
Total	100%	31%	88%	97%	44%

Difficulties in managing menstrual hygiene for physical and intellectual impairments

However, further interactions with mothers of adolescent girls with disabilities and women leaders revealed that menstrual hygiene is difficult to manage for some types of disabilities, for example intellectual. Adolescent girls with disabilities may forget to remove the pad after use or they may fight to remove the pad during the menstrual period. Similarly, for girls with physical impairments, particularly limb deficiencies of the hands, they experience challenges to remove and wash the reusable cloth pads and they have to call for support from parents and are often ignored. This is evidenced in the narration below;

*..And when it is time for my menstruation period, you find that I am using one hand because I have a problem with my other hand, to wash the re-usable sanitary pads and when I tell my mom that I need sanitary pads, she will just ignore by telling me to go away. For instance, when you are in menstrual period, you can be able to use the pads that just removed that day and when you ask you mother to give you money for the sanitary pads, she will just tell you to go away (ignore) even when you tell her to help with washing the pads, she will say, I can't touch your blood things. **Girls with disability, FGD.***

Similar thoughts were echoed by Women Leaders as espoused in the following narrative,

There is this type of disability, those who have got mental illness. It is very difficult for them because they forget to remove the pamper. The time they put that pamper, they forgot. So, they wear it the whole day. So, for that one, who is supposed to help to change. yeah, it's not easy for every disability. Some of them know how to treat themselves, clean and others need help.

Culture of silence compounded by the context of emergency and disability affects information to adolescent girls on menstrual hygiene.

From the interactions with Women leaders and girls with disabilities, it was further revealed that parents rarely talk to their children particularly girls on menstrual health and hygiene. A culture of silence was deemed to be responsible for limited interaction on menstrual health and hygiene management as explained in the narration provided.

Some people don't tell their kids about menstrual hygiene because of our culture. **Women Leader, Local NGO.**

In addition, the disintegration of the traditional family set-up due to emergency made matters worse as family members could help facilitate menstrual health and hygiene and management to adolescent girls. Traditionally, hygiene talks used to be facilitated by close relatives such as aunties. As such, adolescent girls mostly get information on menstrual health and hygiene management from peers.

Yeah, they don't, there are many parents who don't tell their kids. For instance, when we were growing up, they used to send us to our Aunties and your mother can't tell you that, you see; when you do this you, no. They can't, it's hard for her to tell her kids.

...But here we don't have Aunties and that's the problem. **Women Leader, local NGO.**

2.4. Maternal Health

The study revealed that basic services for maternal health are available at Dzaleka refugee camp. However, complicated treatments and surgeries are usually referred to Dowa District Hospital and Kamuzu Central Hospital in Lilongwe.

Findings further reveal that pregnant women and girls with disabilities seek antenatal services very late. As such, they tend to receive the vaccinations late or they may just receive the last vaccination. Sometimes women with disabilities are questioned why they fell pregnant in the first place before giving them the required services. At times, women and girls with disabilities are left with limited choice on the nature of delivery they would want whether vaginal or Caesarean section as they are mostly referred to other hospitals with limited consultation.

That's why we told you that they (pregnant women with disabilities) go at the last minute so that they get one. But it's not like the way they are supposed to go. Firstly, they are shy, they don't want people to know that they're pregnant. You just see them for I think 7 months. They hide themselves. **Women leaders, local NGO.**

*...they don't go to the hospital because when they go, they always blame them saying why did you get pregnant? You know what the situation is like and they don't give them a chance to deliver like other people. They just write a letter to go for a C-section. **Women with disabilities, FGD.***

Furthermore, discriminatory and negative attitudes hugely affect access and delivery of services for women and girls with disabilities. Interaction with girls and women with disabilities revealed that there is negligence by health workers such that women with disabilities may be left to give birth alone with support from their relatives.

*In regards to giving birth, sometimes you go to the hospital and sometimes the way they treat you, when you are going there to give birth and you go there for them to assist you but the nurse will be chatting on the phone until you will start having labor pains and you give birth yourself without assistance. So, the person who came with you from home assists you or maybe yourself. **Girl with disability, FGD.***

Lack of financial resources is a major challenge for maternal health particularly when referral are made without provision of transport from the local health facility.

'Sometimes, you may need medical assistance, but the health service providers will just tell you to go to Dowa Health Centre to get medicines without even providing you with transport. Even if you're pregnant, the treatment is still the same. I remember I was pregnant and I was about to deliver, but when I visited our health centre, they told me to go to Dowa Health centre without providing me with transport, even though the Ambulance was available.' **(FGD Women with disability)**

Lastly, some cultural norms were also seen to be contributing to maternal health issues. There was a belief that women including women with disabilities should continue giving birth so as to have someone to help them in the future as they can rely on children from their relatives. Additionally, women with disabilities also feel pressured to have children when they have remarried as a form of marriage strengthening since the man will not be happy to only be helping the children of the woman from previous relationships.

These findings point to the need to address systemic structural barriers on attitudes of personnel and health workers and also explore empowerment programs for women to have control of their sexual lives.

2.5. Intimate Partner Violence

There are alarming rates of abusive behavior in relationships, particularly highlighting the heightened vulnerability faced by individuals with disabilities. The cases of abuse included emotional, physical abuse and sexual abuse among individuals with and without disabilities.

Emotional Abuse

Both groups of women reported similar incidences of humiliation, with 23% of those without disabilities and 21% of those with disabilities experiencing such treatment. Notably, a higher percentage (29%) of respondents with disabilities reported being insulted or made to feel bad about themselves compared to 23% of those without disabilities.

Physical abuse

Physical aggression is also present, with 22% of individuals with disabilities reporting being pushed, in contrast to 15% without disabilities. The data indicates a concerning prevalence of issues such as threats of violence, with 17% of those with disabilities and 15% of those without reporting threats to harm.

I found someone who was willing to help my kids but that man will always come there to fight with me every day. **Woman with disability, FGD.**

Sexual Abuse

Furthermore, 21% of individuals without disabilities and 23% of those with disabilities indicated being physically coerced into unwanted sexual acts. Other forms of sexual abuse included rape and defilement.

When I was doing my work, a certain man greeted me and accepted and I did not realise who it was and he forced me to sleep with him and I got another pregnant and had another child after this one. **Woman with disability, FGD.**

I reached this place in 2007 and I was still young. I was accompanied by a certain woman because I have no parents. I remember one day, the one who was living with me went to the market and I remained home alone, a man came and raped me and I got pregnant. So, I was too young and decided to keep it

a secret because I knew that if I told that story, they would chase me out of the house. **Woman with disability, FGD.**

Abandonment after marriage

Further interaction with respondents indicated that women with disabilities are usually abandoned by partners after getting pregnant. Interactions with women leaders, the police and women leaders revealed that men coerce women with disabilities into marriage as a means to resettlement, free housing and accommodation. Conversely, women with disabilities consider the marriage advances by men as a form of validation of their worth hence resort to pursue the relationship. However, when the women get pregnant, they are usually abandoned with limited child care support.

... Mostly men are just after accommodation and the rations that women with disabilities get like food. This is because women with disabilities are not left out, they receive these and on top there is resettlement. Local law enforcement agency.

Other issues we face, as women you will find that when you get pregnant men run from the responsibility, because of the disability and another man comes again and impregnates you... Women with disabilities, FGD women.

Through the interactions, it was also noted that community members, particularly family members of men, have negative attitudes towards marriage of men without disabilities to women with disabilities.

When you get pregnant, it's when they (men) usually run. And then the family of that man will come in to say; why are you falling in love with this woman with a disability, don't you see other women? Women with disability, FGD.

Table 3: Summary of examples of violence

Has your partner ever done the following unpleasant acts?	Without Disability	With Disability
Say or do something to humiliate you in front of others?	23%	21%
Threaten to hurt or harm you or someone you care about?	15%	17%
Insult you or make you feel bad about yourself?	23%	29%

Push you	15%	22%
Slap you?	8%	13%
Twist your arm or pull your hair?	8%	8%
Punch you with his fist or with something that could hurt you?	8%	4%
Kick you, drag you or beat you up	8%	8%
Try to choke you or burn you on purpose?	0%	0%
Threaten or attack you with a knife, gun or other weapon	0%	4%
Physically force you to have sexual intercourse with him when you did not want	23%	21%
Force you with threats or in any other way to perform sexual acts you didn't want	15%	8%

Where they report cases of abuse and violence and levels of satisfaction

Respondents reported that they knew several organisations where they report cases of violence including the police, Plan Malawi and World Food Program and some local organisations.

*Am talking on behalf of people with disabilities some of us are living with insecurities, the problem I reported to XX organisation, someone came to my house and tried to force me to have sex, I called the zone leader and I reported the case and the person tried for 2 times forcing me to have sex and I don't know the person. **Woman with disability, FGD.***

However, sentiments shared by the respondent indicated that some people rarely report cases of violence for fear of being removed from the list of beneficiaries for the resettlement program.

*The other barrier is this thing of UNHCR. Many people think, if you report, they will stop your resettlement case. They know when you go to court, that's how it works. They will stop your case until when the case is done, that's when you are... **Women Leader, Local NGO.***

*Someone can do something to you but you just leave because they tell you the more you've got many cases, that's how even in your (resettlement) case by UNHCR no one will touch it because you have got many (court) cases. They told us every Monday, UNHCR goes to the Police to check how many refugees came to report.... So, those files go to them. So, you just leave the thing because you don't want your (resettlement case to be stopped. It's worse. **Women Leader, Local NGO.***

Limited satisfaction with the law enforcement agencies

Respondents' sentiments indicate falling trust in the law enforcement and protective agencies. Respondents indicated concerns of the law enforcement and protective agencies demanding money to fast-track cases. For example, some respondents had this to say;

*...Then I went to the Dzaleka police station and Plan Malawi. Plan Malawi gave me a paper and told me to go to the camp administrator and the camp administrator referred me to Police and they referred me to Police. It's been a week now that I have been visiting the Police and they are asking me for K50,000.00 so that they should give me a security paper. Now, I see that I don't have any help and my help is only cash for ration from the World Food program. **Woman with disability, FGD.***

*...after that I got another man, he gave me another pregnancy. That man, his mother, after knowing that I was pregnant with his son, she did not accept it and then after that I went to plan Malawi and I told them about it. His mother used to come and beat me and they said, we know is not our child son, who is your man? I kept quiet. After that he sent someone to kill my baby and me. After that I went to Plan Malawi and they told me to report at the court. Then I was about to go to court at Dowa. To my surprise, two days before I wanted to go to Dowa, I received some papers and Plan Malawi called me to go. There, they told me that these papers were my order protection. It meant that I was the one harassing them. So, because I'm nothing and I don't have any money, I kept quiet and I didn't go anywhere. **Woman with disability, FGD.***

The findings reveal the need to safeguard access to justice by women who are vulnerable and have gone through violence. Women with disabilities require the law enforcement agencies to be a safe haven where they can find help in time of need.

3. Conclusions and recommendations

3.1. Summary and conclusions

The situation analysis has shown that refugee contexts, gender and disability interact to create multiple deprivations for women even in the context of SRHR. There is therefore a need for inclusive SRHR programming that recognises the intersectionality of the three identity markers

3.2. Recommendations

The recommendations shared here stem from a combination of respondent's insights on what should be done to improve the SRHR design and delivery of services as well as and the consultants' analysis of findings. These have been presented in Table 4.

Table 4: Recommendations for improving delivery of SRHR services

Recommendation	Responsible personnel
Knowledge and Sources of Information on SRHR	
Developing and enhancing outreach efforts that build on peer networks and community health initiatives to improve SRHR knowledge	UPD and health stakeholders
Develop a Comprehensive and Inclusive Sexual Reproductive Health Education Guide	UPD and health stakeholders
Recruit Community volunteer outreach workers to support with outreach programs	UPD
Access to SRHR services	
Develop an inclusive education program on SRHR targeting young women and girls.	UPD
Lobby and mobilise resources to open up affordable community	UPD

clinic	
Conduct capacity building of health personnel in inclusive sexual and reproductive health service delivery.	UPD
Establish and train 'mother group model' on SRHR- to support with adolescent girls with sex education	UPD
Attitudes, norms and practices on SRHR	
Develop a Social Behavioural Change and Communication strategy (SBCC) that takes into account intersectionality of gender and disability	UPD
Recruitment traditional birth attendants, community leaders, and training them on SRHR to support conservative communities	UPD
Champion establishment of a camp health committee or technical working group involving all NGOs and CBOs operating in the camp. UPD could champion the cause for the protection of girls and women with disabilities at the technical group level to leverage on partnerships and resources	UPD
Prevention of Violence and Access to Justice system	
Champion for more resources towards a front-line justice system approach that would utilise <i>justice shelters</i> to expedite cases of women and girls in the camp	UPD and Police

Appendices

Appendix 1: Data collection tools

Tool 1: KII and Focus Group Discussions Guide

Key health challenges for communities (access and use of services)

- a. What are the key health challenges that this community faces?

After response, probe on access and use of services related to:

- a. Maternal Newborn and Child Health, Family Planning and Reproductive Health.
- b. What are the key health services available in this community?
- c. What are the barriers to people accessing health services in this community? Probe for disability related factors on:
- a. Maternal Newborn and Child Health, Family Planning and Reproductive Health
- d. To those who are able to access health services in this community, what facilitates this access? (enablers) Probe on:

Which groups of people easily access these services? Why?

Violence

- e. What types of violence do women and girls with disabilities face in this community and why?
- f. Where do victims of violence (girls/ women) find help in this community?
- g. Why do girls and women fail or delay to seek help?

Menstrual hygiene

- h. What are some of the menstrual hygiene challenges being faced by girls/women with disabilities? (probe access to pads)

- i. How does menstruation affect the lives of girls with disabilities?

Family planning and reproductive health

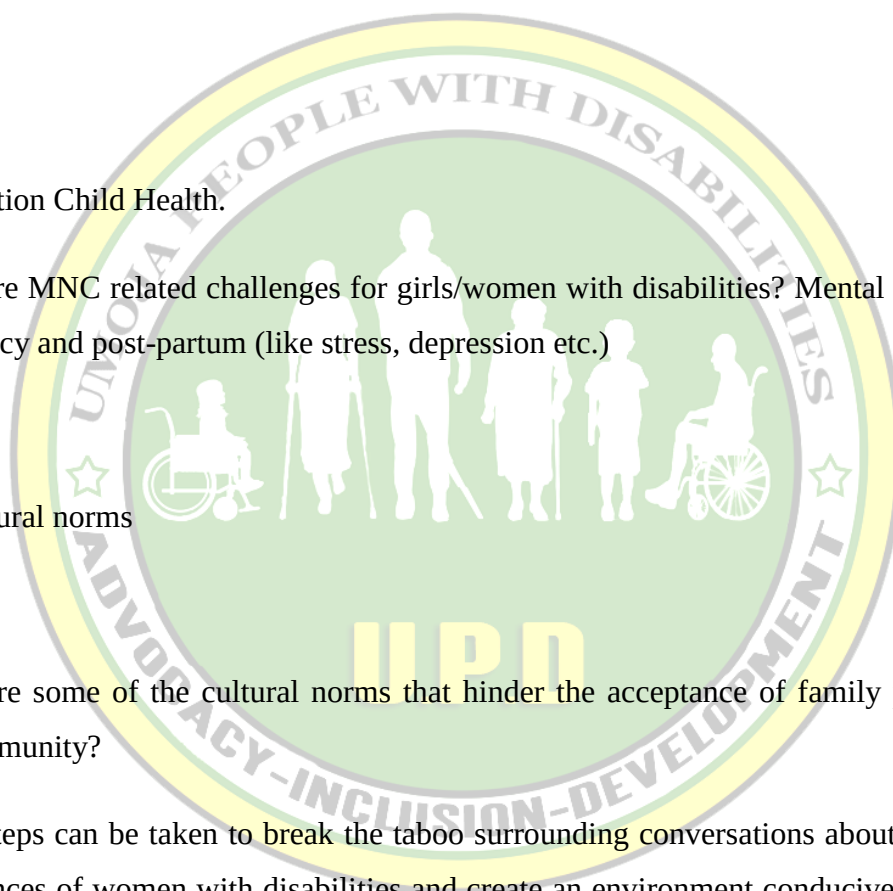
- j. What are the specific challenges faced when accessing FP & RHS? Probe on community, family, service provider challenges)
- k. Why are girls/women with disabilities not accessing SRHS (family planning)?

Maternal Nutrition Child Health.

- l. What are MNC related challenges for girls/women with disabilities? Mental health care issues during pregnancy and post-partum (like stress, depression etc.)

Social and cultural norms

- m. What are some of the cultural norms that hinder the acceptance of family planning services within the community?
- n. What steps can be taken to break the taboo surrounding conversations about family planning, sexual experiences of women with disabilities and create an environment conducive to open dialogue and access to crucial information?
- o. What are some of the assumptions or stereotypes around women/ girls with disability and their sexual & reproductive health SRHS?
- p. What other factors influence women /girls with disabilities with choices regarding their sexual health and contraception choices?



Tool 2 :KII Guide, Police

- a. What are some of the common crimes committed against girls and women as recorded by your office? Probe on girls and women with disabilities.
- b. Are there any cultural/religious beliefs that shape issues of violence against girls and women with disabilities in this community?
- c. On violence against girls/women with disabilities, how do you support victims?
- d. How inclusive is the victim support unit when dealing with girls/women with disabilities?
- e. Some women have told us that they prefer to keep quiet/not report when offended for the fear of having many cases at the police which affects their resettlement ...what is your comment?
- f. Are there any special programs on public education that focus on the protection of girls and women with disabilities?
- g. How do you work with other stakeholders in the camp in order to help girls and women with disabilities?
- h. How best can girls and women with disabilities be supported to report abuse/violence?

Tool 3 :KII Guide, Health Personnel

Key health challenges for communities (access and use of services)

1. What are the key health challenges that this community faces?

After response, probe on access and use of services related to:

1. *Maternal Newborn and Child Health, Family Planning and Reproductive Health.*
2. What are the key health services available in this community?
3. What are the barriers to people accessing health services in this community? Probe for disability related factors on:

1. *Maternal Newborn and Child Health, Family Planning and Reproductive Health*

4. To those who are able to access health services in this community, what facilitates this access?
(enablers) *Probe on:*

Which groups of people easily access these services? Why?

- e. *On family planning, what types of FP services do you offer and how inclusive is the delivery of these services?*
- f. *What are the specific challenges faced when delivering FP & RHS? Probe on community, family, service provider challenges)*
- g. *Why are girls/women with disabilities not accessing SRHS (family planning)?*
- h. *On maternal health, what are some of the MNC related challenges for girls/women with disabilities? WHY?*
9. *What are some of the cultural norms/beliefs that affect utilisation of SRHS for women and girls with disabilities?*

Tool 4 :KII Guide- Single Mothers

- a. What factors necessitated the establishment of single mothers' association?
- b. What are some of the common challenges being experienced by girls and women in the camp? Probe on girls and women with disabilities.
- c. Are there any cultural/religious beliefs that shape issues of violence against girls and women with disabilities in this community?
- d. On violence against girls/women with disabilities, how do you support victims?
- e. Are there any special programs on public education that focus on the protection of girls and women with disabilities?
- f. How do you work with other stakeholders in the camp in order to help girls and women with disabilities?

g. How best can girls and women with disabilities be supported to report abuse/violence?



Appendix 2: Study terms of reference

TERMS OF REFERENCE TO CONDUCT OPERATIONAL RESEARCH ON SEXUAL AND REPRODUCTIVE HEALTH AND RIGHTS TOWARDS AN EFFECTIVE INCLUSION OF REFUGEES WITH DISABILITIES IN MALAWI.

1. BACKGROUND

UMOJA PEOPLE WITH DISABILITIES is a non-profit organization, registered TR/INC N° 6686 and working under the law of Malawi. Our mission is to create a world where the rights of refugees with disabilities are respected, and their lives are as comfortable as possible. UPD is a member of the Global Refugee Youth Network (GRYN), the Refugee Organization in Malawi (RELON), and the United Nations High Commission for Refugees (UNHCR). Their primary focus has been on empowering persons with disabilities within the Dzaleka Refugee Camp by providing IT skills to enhance job search opportunities. and aiming to raise up the rights of persons living with disabilities in particular and all vulnerable in general within Dzaleka refugee camp and host communities, its missions and goals are to improve the quality of life of people with disabilities, vulnerable and other in need in the community (1), to obtain justice through equalization of opportunities for all people with disabilities through the development and support of their associations (2), to fight against poverty among the persons with disabilities (3), to promote and advocate for the Rights of persons with disabilities and raise awareness among the rural masses in the communities on these rights (4) etc.

With Financial support from the Disability Rights Fund (DRF), UPD is commissioning a study to conduct a needs assessment survey by interviewing refugees with disabilities to identify specific SRH needs and barriers and systematically analyse the importance of sexual and reproductive health and rights (SRHR) in achieving Universal Health Coverage (UHC). While there have been some publications highlighting the importance of SRHR for UHC, there is a lack of comprehensive research that makes a compelling case for its integral role. UPD through this study wants to invest in systematic operational research to illustrate why and how SRHR is an integral component for UHC as well document the role of civil society organisations like UPD's to achieve UHC locally, nationally and globally.

Sexual and reproductive health and rights (SRHR) are fundamental human rights related to sexuality and reproduction. Access to SRHR is essential throughout a person's life, starting with comprehensive

sexuality education in childhood programs to eliminate female genital mutilation, and efforts to prevent early marriage. This extends through their reproductive years and into later life, including the prevention of reproductive cancers. When individuals can make informed choices about sex and reproduction and are able to lead safe and healthy sexual and reproductive lives, they are better positioned to participate in education and the workforce, care for their families, and contribute more effectively to their communities and social well-being. Therefore, SRHR information and services must be an integral part of any core package of essential health services that are universally accessible to all.

2. Purpose of the Research:

The purpose of this research is to conduct a comprehensive needs assessment to identify the specific sexual and reproductive health (SRH) needs and barriers faced in accessing SRH services by women and girls with disabilities at Dzaleka Refugee Camp in Malawi. The research aims to gather data on the unique challenges this population encounters in accessing SRH services, education, and information, and to explore the impact of disability, gender, and refugee status on their sexual and reproductive health outcomes. The findings will provide critical insights into the gaps in services and the socio-cultural, physical, and logistical barriers that prevent women and girls with disabilities from fully accessing SRH care. Ultimately, the research will inform the development of targeted interventions and advocacy strategies to improve SRH services and outcomes for this marginalized group.

3. Scope of the Terms of Reference (ToR):

1. Target Population

Women and girls with disabilities residing in Dzaleka Refugee Camp, with a focus on their SRH needs, access to services, and experiences.

2. Research Objectives:

- To identify the specific SRH needs of women and girls with disabilities, including but not limited to access to contraception, maternal health care, menstrual hygiene, prevention of sexual violence, and sexual health education
- To assess the physical, social, and economic barriers these women and girls face in accessing SRH services and information.
- To evaluate the role of community attitudes, cultural norms, and institutional practices in shaping access to SRH care for women and girls with disabilities.
- To gather recommendations for improving SRH service delivery and support for women and girls with disabilities at the camp.

- Secure the formal inclusion of disability-inclusive sexual and reproductive health (SRH) services in national health policies and Dzaleka Refugee Camp's health management strategies for refugees with disabilities.
- Increase the accessibility and quality of sexual and reproductive health (SRH) services for refugee women and girls with disabilities at Dzaleka Refugee Camp in Malawi.
- Safe health services access and people of concern are centered care mobilization

3. Data Collection Methods:

- Surveys, focus group discussions (FGDs), case studies and in-depth interviews with women and girls with disabilities, caregivers; and key informant interviews with community leaders and service providers within the camp.
- Review of existing reports, documents, and policies related to SRH services and disability inclusion in the camp.

4. Geographical Focus

Dzaleka Refugee Camp, located in the Dowa District of Malawi, including surrounding host communities where applicable.

5. Duration of the Research

The research will be conducted over a period of 6-8 weeks, including data collection, analysis, and report writing.

6. Key Deliverables

- A comprehensive needs assessment report detailing the findings on the SRH needs and barriers for women and girls with disabilities in Dzaleka refugee Camp.
- Practical recommendations for improving access to SRH services and support for this group.
- A summary report for dissemination to relevant stakeholders, including government agencies, NGOs, and community organisations working on disability and SRH issues.

7. Expected Outcomes

- Increased awareness among stakeholders (government, service providers, NGOs) about the specific SRH needs and challenges faced by women and girls with disabilities at the camp.
- Development of actionable recommendations to enhance SRH service provision and policies that are inclusive of women and girls with disabilities.

- Strengthened advocacy efforts for improving SRH outcomes and disability inclusion in refugee contexts.

KEY COMPETENCES, TECHNICAL BACKGROUND, AND EXPERIENCE REQUIRED

- Excellent and demonstrated experience in undertaking desk-based robust literature research
- Proven experience in research and analysis
- Proven experience in report writing, editing with strong track record of producing similar publications for dissemination
- Demonstrated experience of publications in peer reviewed journals
- Experience in and commitment to working in sexual and reproductive health and rights fields
- Substantial knowledge of global health policy, health financing and health economics, especially health systems strengthening and universal health coverage
- A good understanding of the target audience this publication is aimed at as specified under section.
- Excellent writing, research and analytical skills
- Experience preparing and presenting clear and concise oral and written communications
- Ability to analyze data and summarize information and innovatively depict information such as infographics, etc.

To apply, please submit a cover letter including your motivation to apply, daily rate and methodology/ search strategy proposed including timeline; your CV (indicating experience of research, systematic reviews and publications in peer review journals; a sample of a publication where you have been a lead or co-author, etc).

The closing date for applications is February 20, 2025 and the consultancy to be agreed no later than March 15, 2025

